

UK Legal & Compliance Pack

Practice-management platform for therapists · Faresay as data processor, therapist as controller

DRAFT for professional sign-off. These are working drafts prepared to support a UK launch. They are not legal advice and must be reviewed and finalised by a qualified solicitor before reliance. Placeholders marked [PLACEHOLDER] require completion.

11 documents

Client & customer terms

1. Privacy Policy
2. Terms of Service
3. Therapist Agreement

Data protection (processor model)

4. Data Processing Agreement (UK GDPR Art 28)
5. Technical & Organisational Measures (Annex 2)
6. Sub-processor List (Annex 3)
7. Record of Processing Activities (Art 30)
8. Data Protection Impact Assessment (Art 35)

Operational policies

9. Security & Data Protection Policy
10. Clinical Governance Policy
11. Crisis & Safeguarding Policy

Privacy Policy

⚠️ DRAFT v0.2 — for legal/privacy counsel review. NOT legal advice. Counsel must finalise before use. Last updated: 26 June 2026

Faresay Privacy Policy

Faresay is **practice-management software for therapists**. UK-registered therapists, practitioners and practices subscribe to Faresay to run their own private practice — managing their own client list, scheduling and conducting secure video sessions, sending reminders, and taking card payments. Our customer is the therapist. Faresay is **not** a marketplace, a directory, a matching or client-acquisition service, or a healthcare provider.

This Privacy Policy explains how **Faresay** handles personal data in its own right — principally the account, billing, security and website data of the therapists who subscribe to us and the visitors to our website. For a therapist's **clients'** data (including special-category mental-health information), Faresay acts only as a **processor** on the therapist's instructions; the **therapist is the controller** of that data. See Section 1.

Because the software is used to deliver mental-health care, some of the data flowing through it is **special-category / sensitive health data**. We treat all such data with a correspondingly high standard of confidentiality and security, whichever role we are in.

This policy should be read alongside our [Terms of Service](#), the [Faresay Therapist Agreement](#), the [Data Processing Agreement](#), the [Sub-processor List](#), and our [Security & Data Protection Policy](#).

1. Who we are & our two data-protection roles ⚠️ COUNSEL

Faresay ("Faresay", "we", "us", "our") provides the practice-management software that an independent, UK-registered therapist uses to run their own private practice. The therapist brings their own clients, owns the client relationship and the clinical record, and decides why and how their clients' data is used. Faresay provides the tools.

- **Legal entity:** Faresay Ltd
- **Registered address:** 167-169 Great Portland Street, London W1W 5PF, England & Wales
- **Company registration number:** 17302034
- **ICO registration number (UK):** applied for 26 June 2026 — registration number to follow
- **Data Protection contact:** Joel Harvey, Founder — support@faresay.com

Faresay holds **two distinct roles** under UK GDPR, and it matters which one applies to a given piece of data:

- **Faresay as controller.** For the **therapist's** own account, identity, practice, billing, subscription, usage, support and security data — and for data about **visitors to our website** — Faresay is the **controller**. **This Privacy Policy applies to that data.**
- **Faresay as processor.** For a therapist's **clients'** data — including intake responses, scheduling, messages and special-category mental-health information — Faresay is a **processor**, acting only on the therapist's

documented instructions under our [Data Processing Agreement](#) (UK GDPR Article 28). The **therapist is the controller** of that client data.

Faresay does **not** hold or own the clinical record and does not decide the purposes for which a therapist's clients' data is used.

If you are a client of a therapist who uses Faresay: this is not the right document for you. Please refer to **your therapist's own privacy notice**, and contact your therapist to exercise your rights over your data. Faresay will assist your therapist as required under the Data Processing Agreement.

⚠ COUNSEL — confirm the controller / processor split. Counsel must confirm that, under UK GDPR and the Data Protection Act 2018, (a) Faresay is the **independent controller** of the account-administration, billing, security and website data described in Section 3, and (b) Faresay is the **processor** (and the therapist the **controller**) of clients' clinical/personal data, consistent with the [Data Processing Agreement](#) and the [Therapist Agreement](#). The roles must be reflected consistently across this policy, the Terms, the Therapist Agreement and the DPA.

2. Scope (UK-first)

This policy applies to Faresay's services as offered in the **United Kingdom**, governed by the **UK GDPR** and the **Data Protection Act 2018**.

It covers our website, the Faresay web and mobile applications, and related services (together, the "**Platform**" or the "**software**") **in respect of the data for which Faresay is the controller** (see Section 1). It does **not** govern a therapist's processing of their own clients' data — that is the therapist's responsibility as controller, supported by the [Data Processing Agreement](#).

This policy does **not** cover third-party websites or services we link to, which have their own privacy practices.

⚠ COUNSEL — Faresay is a UK-first product. If and when Faresay serves customers outside the UK, or engages sub-processors that change the transfer picture, counsel should confirm whether a market-specific or supplemented notice is required before any non-UK launch.

3. What data we collect (as controller)

We collect the following categories of personal data **about therapists who subscribe to us and about website visitors**. (A therapist's clients' intake, scheduling and clinical data is **not** covered here — Faresay processes that only as a processor under the DPA; see Section 1.)

3.1 Account & identity data

- Name, email address, telephone number, and login credentials (passwords stored hashed via our authentication provider).
- The therapist's professional identity: practice/business name, professional registration body, registration number, and verification information.
- Profile and account-configuration information the therapist provides.

3.2 Billing & subscription data

- Subscription plan, transaction history, and records of the **monthly subscription fee** and the **card-processing commission**.
- Billing details and invoices.
- Payment is processed by our payment provider; we do **not** store full card numbers on our own systems.
[PLACEHOLDER: confirm exactly what billing data Faresay stores vs. what the payment processor stores.]

3.3 Usage & device data

- IP address, device identifiers, browser type, operating system.
- Log data, pages/screens viewed, features used, and timestamps.
- Security telemetry and audit logs.

3.4 Communications data

- Messages you (the therapist) send to us (support, enquiries) and our responses.
- Feedback, survey responses and product correspondence.

3.5 Marketing data

- Contact details and marketing preferences, where you have opted in.

We collect this information directly from the therapist, automatically through use of the software, and in some cases from third parties (e.g. professional-registration or identity-verification providers, and our payment provider regarding transactions).

Not covered here: a therapist's clients' intake responses, scheduling, messages and **special-category health data**. Faresay processes that data **only as a processor** on the therapist's documented instructions under the [Data Processing Agreement](#). The therapist is the controller of it.

4. How and why we use your data, with legal bases (as controller)

The table below summarises the main purposes for which **Faresay, as controller**, uses therapist and website-visitor data, and the **legal bases** we rely on under UK GDPR.

⚠ COUNSEL — the legal bases below are a first draft and must be confirmed. Note that, as controller, Faresay is **not** relying on these bases to process clients' special-category health data — that processing sits with the **therapist as controller** under the DPA, on the conditions described in Annex 1 of the DPA (UK GDPR Art 9(2)(h) and/or Art 9(2)(a)).

#	Purpose	Data used	UK GDPR Art 6 basis
1	Create and manage the therapist's account and provide the software	Account/identity, usage	Contract (Art 6(1)(b))
2	Charge and collect the monthly subscription and card-processing commission	Billing/subscription	Contract (Art 6(1)(b)) / Legal obligation (accounting)
3	Verify professional registration and eligibility to use the software	Account/identity, verification	Legal obligation / Legitimate interests ⚠ COUNSEL

#	Purpose	Data used	UK GDPR Art 6 basis
4	Provide customer support and respond to enquiries	Communications, account	Contract / Legitimate interests
5	Security, fraud prevention, and protecting the software	Usage/device, account	Legitimate interests / Legal obligation
6	Product analytics and service improvement (aggregated/minimised where possible)	Usage/device	Legitimate interests
7	Marketing communications to therapists and prospects (where permitted)	Account, marketing	Consent / Legitimate interests
8	Legal compliance, regulatory and dispute handling	As needed	Legal obligation / Legitimate interests (Art 9(2)(f) for legal claims where any special-category data is involved)

We will not use the controller data we hold for a new, incompatible purpose without identifying an appropriate lawful basis and, where required, obtaining your consent.

5. How we share your data

We share personal information only as described below.

- **Service providers / processors & sub-processors.** We use third-party providers to host, operate and support the software (e.g. cloud hosting and database, authentication, payment processing, video delivery, email/SMS, and privacy-focused analytics). They process data on our instructions under written contracts. The current list is in the [Sub-processor List](#). For controller data they act as our processors; for a therapist's clients' data they act as our sub-processors under the [Data Processing Agreement](#).
- **Payment provider.** Our payment processor handles the subscription, the card-processing commission and the therapist's clients' card payments, under its own terms; card numbers are tokenised by the provider and not stored by Faresay. [PLACEHOLDER: name of payment provider — see sub-processor list.]
- **Legal & safety.** We may disclose information where necessary to comply with law, respond to lawful requests, enforce our Terms, prevent fraud, or protect the rights, safety and vital interests of any person.
- **Business transfers.** If Faresay is involved in a merger, acquisition, financing or sale of assets, personal information may be transferred as part of that transaction, subject to appropriate confidentiality and continued protection of your data.

We do **not** sell your personal information, and we do **not** use a therapist's clients' data for our own purposes, profiling, marketing or to train AI models (see DPA Section 3).

6. International data transfers ⚠️ COUNSEL

Our preferred residency for personal data is **UK or EEA** regions. Some of our sub-processors are US-based. Where we transfer personal data outside the UK, we put in place an appropriate safeguard, which may include:

- the **UK International Data Transfer Agreement (IDTA)** or the **UK Addendum** to the EU Standard Contractual Clauses (**SCCs**);

- the **EU Standard Contractual Clauses** for EEA-origin transfers;
- reliance on an **adequacy** regulation, where applicable; or
- other lawful transfer mechanisms,

supported, where required, by a transfer risk assessment. The specific safeguard for each sub-processor is recorded in the [Sub-processor List](#).

⚠ **COUNSEL** — confirm the transfer mechanism for each data flow, confirm current adequacy status, and complete transfer risk assessments where required. Given the sensitivity of the mental-health data flowing through the software, transfer safeguards warrant particular scrutiny.

7. Data retention ⚠ **COUNSEL**

We keep **controller** personal data only for as long as necessary for the purposes set out in this policy, and to meet legal, regulatory, accounting and dispute-resolution requirements.

- Account & identity data: [PLACEHOLDER: retention period — typically for the life of the subscription plus a short tail]
- Billing & subscription data: [PLACEHOLDER: retention period — typically ~6 years for UK tax/accounting law]
- Usage, device & log data: [PLACEHOLDER: retention period]
- Communications & support data: [PLACEHOLDER: retention period]
- Marketing data: [PLACEHOLDER: retention period / until opt-out]

When controller data is no longer required, we securely delete or anonymise it.

Clients' data. Faresay does **not** set or hold the retention period for a therapist's clients' clinical data. That is governed by the therapist's own clinical-records obligations and the [Data Processing Agreement](#): the therapist can export their data at any time, and on termination Faresay returns or deletes client data on the therapist's instruction (see DPA Section 11).

⚠ **COUNSEL** — finalise the controller-data retention schedule, and confirm it is consistent with the return/deletion mechanics for client data in the DPA.

8. Security

We implement technical and organisational measures designed to protect personal information against unauthorised access, loss, misuse or alteration, with enhanced controls appropriate to the sensitivity of the mental-health data flowing through the software. These measures are described in our [Security & Data Protection Policy](#) and the [Technical & Organisational Measures](#), which this policy incorporates by reference.

No system can be guaranteed 100% secure, but we maintain a high security bar appropriate to the sensitivity of the data.

9. Your rights

9.1 Therapists and website visitors (Faresay as controller)

Where Faresay is the **controller** of your data, and subject to applicable law, you have the right to: - **access** the personal data we hold about you; - request **rectification** of inaccurate data; - request **erasure** ("right to be forgotten") in certain circumstances; - **restrict** or **object** to certain processing; - **data portability**; - **withdraw consent** at any time where processing is based on consent (without affecting prior processing); and - not be subject to solely automated decisions with legal/similarly significant effects (see Section 12).

To exercise these rights, contact us using the details in Section 14. We will respond within the timeframes required by law. ⚠️ **COUNSEL** — note that some rights (e.g. erasure) may be limited where data must be retained for legal, accounting or regulatory reasons.

9.2 Clients of a therapist

If you are a **client** of a therapist who uses Faresay, the therapist is the **controller** of your data. Please contact **your therapist** to exercise your rights (access, rectification, erasure, restriction, portability, objection). Faresay, as processor, will assist the therapist in responding, and will refer any request it receives directly to the therapist without undue delay (see DPA Section 7).

10. Cookies & analytics

We keep tracking to a minimum.

- We use **strictly necessary** cookies only — for example, to authenticate you and keep the software secure. These are required for the software to function.
- For website analytics we use a **cookieless, privacy-focused** analytics provider that does not set cookies, does not collect personal data, and does not track you across sites (see the [Sub-processor List](#)).
- We do **not** use advertising trackers.

⚠️ **COUNSEL** — confirm the cookie classification and that no non-essential cookies are set without consent where required (PECR / UK law). [PLACEHOLDER: link to detailed cookie notice / preference centre, if one is required.]

11. Children's data

Faresay's **customer is a professional therapist**, and the software is intended for use by adult therapists running their own practice. Faresay does not knowingly collect personal data about children in its role as controller.

A therapist may, as controller, use the software to support clients who are minors. Where that happens, the therapist is responsible, as controller, for the lawful basis, consent/authorisation and safeguarding obligations relating to those clients; Faresay processes that data only as a processor under the [Data Processing Agreement](#).

⚠️ **COUNSEL** — confirm whether the software is offered for use with minors at all, and what additional obligations follow (UK Age Appropriate Design Code; parental/guardian consent; safeguarding) for the therapist as controller and for Faresay as processor. See the [Crisis & Safeguarding Policy](#).

12. Automated decision-making

Faresay does **not** make decisions producing legal or similarly significant effects about you based **solely** on automated processing without human involvement. The software provides tools to the therapist; clinical decisions are made by the therapist, not by Faresay.

⚠️ **COUNSEL** — confirm this remains accurate as the product evolves; if any solely-automated significant decisions are introduced, additional UK GDPR Article 22 safeguards and disclosures will be required.

13. Data breaches ⚠️ **COUNSEL**

We maintain procedures to detect, investigate and respond to personal-data breaches.

- **Faresay as controller.** Where a breach affecting controller data is likely to result in a risk to rights and freedoms, we will notify the **ICO** without undue delay and, where required, within **72 hours** of becoming aware, and affected individuals where the breach is high-risk.
- **Faresay as processor.** Where a breach affects a therapist's clients' data, Faresay will notify the therapist (the controller) **without undue delay, and in any event within 48 hours**, and assist the therapist with their own notification duties to the ICO and data subjects (see DPA Section 8).

Our incident-response approach is detailed in the [Security & Data Protection Policy](#).

14. How to complain ⚠️ **COUNSEL**

If you have a concern about how we handle your personal data, please contact us first (Section 15) so we can try to resolve it.

You have the right to lodge a complaint with the **Information Commissioner's Office (ICO)** — <https://ico.org.uk> — though we ask that you contact us first.

If you are a **client** of a therapist, please raise data concerns with your therapist (the controller) in the first instance; you may also complain to the ICO.

⚠️ **COUNSEL** — confirm the complaints routing between Faresay (controller and processor roles) and the therapist (controller), and the ICO contact details.

15. Contact & updates

Privacy / data-protection contact: Joel Harvey, Faresay Ltd — support@faresay.com **Registered office:** 167-169 Great Portland Street, London W1W 5PF, England & Wales **Data Protection Officer (if appointed):** None appointed — no statutory DPO required at current scale; data-protection contact as above. ⚠️ **COUNSEL** — confirm no Article 37 DPO obligation given the scale and nature of processing.

We may update this Privacy Policy from time to time. We will post the updated version with a revised "Last updated" date and, where changes are material, take additional steps to notify affected therapists as required by law.

This is a DRAFT document prepared for legal/privacy counsel review. It is not legal advice and must be reviewed and finalised by qualified UK counsel before publication or use.

Terms of Service

⚠️ DRAFT v0.2 — for legal counsel review. NOT legal advice. Counsel must finalise before use. Last updated: 26 June 2026

Faresay — Terms of Service

These Terms of Service ("**Terms**") govern your subscription to and use of Faresay's practice-management software, including our website, applications, and related services (together, the "**Service**"). The Service is operated by Faresay Ltd, a company registered in England & Wales (company number 17302034) with registered office at 167-169 Great Portland Street, London W1W 5PF ("**Faresay**", "**we**", "**us**", "**our**").

Please read these Terms carefully. They form a business-to-business agreement between Faresay and you, the mental-health professional or practice that subscribes to the Service (the "**Customer**", "**you**", "**your**"). They apply to Customers in the United Kingdom and, where and when the Service is made available, in other markets.

⚠️ COUNSEL — Confirm the contracting entity for each market (UK operating entity now; any additional entity on later-market launch — see `business-model-canvas.md`). These Terms may need a market-specific or jurisdiction-specific addendum or version. Faresay's relationship to its Customers is that of a software supplier; the previous marketplace/intermediary framing has been removed throughout.

1. Acceptance of these Terms

1.1 By creating an account, subscribing to, accessing, or using the Service, you confirm that you have read, understood, and agree to be bound by these Terms and by any documents expressly incorporated by reference (including our Privacy Policy — see `privacy-policy.md` — and the Data Processing Agreement — see `data-processing-agreement.md`). If you do not agree, you must not use the Service.

1.2 You confirm that you are subscribing **in the course of a business** (whether as a sole trader, partnership, or company) and not as a consumer, and that you are authorised to enter into these Terms. If you are subscribing on behalf of an organisation, you represent that you are authorised to bind that organisation, and "you" refers to that organisation.

1.3 We may require you to affirmatively accept these Terms (for example, by clicking "I agree") before you can use certain features. Your continued use of the Service constitutes ongoing acceptance.

2. Description of the Service

2.1 **Faresay is practice-management software.** The Service is a software-as-a-service (SaaS) platform that an independent, appropriately registered mental-health professional subscribes to in order to run their own private practice — managing clients, scheduling appointments, holding secure video sessions, taking card payments, sending reminders, and keeping practice records.

2.2 **Faresay does NOT provide clinical, therapy, counselling, medical, or healthcare services.** Faresay is **not** a healthcare provider, a medical practice, a provider of mental-health treatment, a directory, a matching or referral service, or a marketplace. Faresay does not practise psychology, psychotherapy, counselling, social work, or medicine, does not supply, introduce, or acquire clients for you, and does not supervise, direct, or control your clinical judgement.

2.3 **You bring your own clients and run your own practice.** The clinical relationship, the client relationship, and the clinical record are **between you and your client**. Your clients are your clients, not Faresay's. Faresay is not a party to your engagement with any client and does not hold or own the clinical record. See Section 6.

2.4 The Service Faresay provides is **software and supporting technology**: tools for client and contact management, scheduling, secure messaging, video-session hosting, reminders, card-payment processing, and practice records. ⚠️ COUNSEL — Faresay's role is that of a software supplier and (for the payment facility) a technology provider, not a clinical, referral, or intermediary service; confirm the wording aligns with the product as built (see `product-requirements-document.md`).

2.5 We do not guarantee that the Service will meet every need of your practice, or that any particular feature will be available indefinitely. We may add, change, or remove features over time, and will give reasonable notice of any material reduction in functionality.

3. NOT AN EMERGENCY OR CRISIS SERVICE

⚠️ THE SERVICE IS NOT FOR EMERGENCIES OR CRISES.

Faresay and the Service are NOT an emergency service, crisis line, or suicide-prevention service. The Service is practice-management software for scheduled, non-urgent care delivered by you. Faresay does **not** monitor client communications or bookings for emergencies, and messages or bookings made through the Service are not monitored on your behalf.

If you or a client is experiencing a medical or mental-health emergency, is in crisis, or is at risk of harm, do NOT rely on the Service. Get help immediately:

United Kingdom - Call **999** for any life-threatening emergency - Call **NHS 111** (select the mental-health option) for urgent help - Call the **Samaritans** free on **116 123** (24 hours a day) - Text **SHOUT** to **85258** (free, 24/7)

If you are outside the United Kingdom, contact your local emergency services.

⚠️ COUNSEL / ⚠️ CLINICAL — Verify all numbers and service descriptions before publication, and confirm they match the `crisis-safeguarding-policy.md` . Confirm 999, NHS 111 mental-health routing, Samaritans 116 123, and SHOUT 85258 details and any region-specific equivalents.

3.1 You acknowledge that the Service supports non-urgent care delivered by you on a scheduled basis, that response times for messages and bookings are not immediate, and that crisis handling, risk assessment, and safeguarding rest with you as the clinician (see Section 6).

4. Eligibility

4.1 The Service is for **professional use** by appropriately qualified practitioners. To subscribe and use the Service to deliver care, you must be a mental-health professional appropriately registered and entitled to practise in the United Kingdom — a current member of an appropriate professional body or PSA-accredited register (for example BACP, UKCP, NCPS, or BPS), or, where your profession is statutorily regulated, holding the relevant statutory registration.

4.2 You must maintain that registration in good standing, hold your own professional indemnity insurance, and comply with your professional body's standards and applicable law throughout your use of the Service.

4.3 By subscribing, you represent and warrant that you meet these eligibility requirements and that the information you provide is accurate. We may ask you to evidence your registration and insurance, and may suspend or

terminate your account under Section 17 if you cease to meet these requirements. ⚠️ COUNSEL / ⚠️ CLINICAL — confirm what registration and insurance checks Faresay performs at onboarding and on renewal (see `clinical-governance-policy.md` and `therapist-agreement.md`); align the representations here with what is actually done.

4.4 Location and licensure. You are responsible for ensuring you are lawfully entitled to provide your services to each client in the place where the client is located at the time of a session, including any cross-border or remote-care requirements. Faresay does not assess or guarantee your entitlement to practise in any location. ⚠️ COUNSEL — confirm wording for any later non-UK markets.

5. Accounts and Registration

5.1 To use the Service you must create an account. You agree to provide accurate, current, and complete information and to keep it up to date.

5.2 You are responsible for safeguarding your login credentials and for all activity that occurs under your account, including the activity of any associate or staff member to whom you grant access. Notify us promptly at [PLACEHOLDER: security/support contact] if you suspect any unauthorised use.

5.3 You may not share your account credentials, create an account using false information, or permit access by anyone not authorised by you. You are responsible for ensuring anyone you authorise complies with these Terms.

5.4 We may refuse, suspend, or terminate accounts in accordance with Section 17.

6. Your Practice and Your Clients

6.1 You run your own practice. As an independent professional, you — and not Faresay — are solely responsible for the mental-health and clinical services you provide, including your professional judgement, the suitability, safety, and quality of care, your compliance with applicable licensing, professional, and ethical standards, and the maintenance, retention, and security of your clinical records.

6.2 The client relationship is yours. Faresay is not a party to the relationship between you and your client. Faresay's role is limited to providing the software you use to run your practice. Your clients are your clients; we do not acquire, introduce, refer, allocate, or match clients to you, and we make no claim to the client relationship or the clinical record.

6.3 Crisis, risk, and safeguarding. You are responsible for crisis handling, risk assessment, safeguarding, and signposting clients to appropriate UK services. The Service is not a substitute for, and does not perform, any of these clinical responsibilities (see Section 3).

6.4 Your engagement documents govern care. Any agreement for clinical services, including your own client terms, informed-consent forms, and notices, is between you and your client. Where there is a conflict between these Terms and your clinical engagement documents on a clinical matter, your clinical documents govern that clinical matter.

7. Telehealth and Remote Care

7.1 The Service supports the delivery of care by remote/telehealth means (for example, video, voice, or messaging). Telehealth has benefits and limitations and may not be appropriate for every client or condition.

7.2 As the treating professional, **you** are responsible for determining whether remote care is appropriate and for obtaining your client's **informed consent to telehealth** as required by applicable law and professional standards,

including explaining the nature, benefits, risks, and limitations of remote care, alternatives, confidentiality, and what to do in an urgent situation. Faresay does not capture telehealth consent on your behalf.

7.3 Faresay provides the technology that enables remote sessions; it does not direct or assess the clinical appropriateness of delivering any particular care remotely.

⚠️ COUNSEL / ⚠️ CLINICAL — Confirm telehealth informed-consent requirements applicable to UK practitioners and that responsibility for capturing consent sits with the Customer (clinician), consistent with `clinical-governance-policy.md`. Do not finalise this section without counsel and a clinical advisor.

8. No Professional Relationship with Faresay; Software Disclaimer

8.1 **No clinician–client relationship is formed with Faresay.** Nothing on or about the Service, and no interaction with Faresay or its staff, creates a clinician–client, doctor–patient, therapeutic, or other professional clinical relationship between Faresay and you or your clients. Our relationship with you is that of software supplier and Customer.

8.2 **Faresay does not provide clinical or professional advice.** Any general information, guidance, or materials Faresay makes available (for example, product help, templates, or best-practice notes) are for general informational purposes only and are **not** clinical, legal, regulatory, or professional advice, and are not a substitute for your own professional judgement. You are responsible for ensuring your practice complies with applicable law and professional standards.

8.3 Faresay does not recommend or endorse any particular treatment, opinion, or course of action in respect of any client. Reliance on any information made available through the Service is at your own risk.

9. Subscription, Fees, and Payments

9.1 **Subscription.** The Service is offered on **monthly subscription plans**, billed monthly in advance through our payment provider, Stripe. The applicable plans, tiers, and prices are disclosed in-product before you subscribe. Your subscription is a **software fee** for access to the Service.

9.2 **Card-payment commission.** Where you use the Service to take card payments from your clients, Faresay charges a **small commission** on each card payment processed, in addition to your subscription fee and the underlying Stripe processing fees. This commission is a **fee for the payment-processing facility within the software**. It is **not** a marketplace fee, a referral fee, a take rate on your practice, or any share, split, or cut of your professional or clinical fee. You set your own fees, own your clients, and receive your fees less only the processor fees and this commission. The commission rate is disclosed in-product before you enable payments. ⚠️ COUNSEL — confirm how the subscription fee and the processing commission are each presented to Customers and that the presentation is consistent with the characterisation in `financial-model.md` and `business-model-canvas.md`.

9.3 **No share of clinical fees.** For the avoidance of doubt, Faresay takes no percentage of, and does not split, your session or clinical fees. Faresay's revenue from you consists only of (a) the software subscription and (b) the card-payment processing commission described above.

9.4 **Payment processing.** Card payments are processed through Stripe (or another third-party payment provider we notify to you). By providing payment details and enabling payments, you authorise us and our payment provider to charge the applicable subscription fees and to deduct the processing commission and processor fees from amounts collected. You agree to the payment provider's terms. ⚠️ COUNSEL — confirm the flow of funds and the payment provider's contracting and settlement model.

9.5 **Taxes.** Prices are exclusive of any applicable taxes (including VAT) unless stated otherwise. You are responsible for any taxes that apply to you, including any VAT chargeable on your own services to your clients. ⚠️

COUNSEL — confirm UK VAT treatment of the subscription and the processing commission.

9.6 Billing changes and non-payment. We may change subscription prices or the commission rate on reasonable prior notice, effective from your next billing period; continued use after the change takes effect constitutes acceptance, and if you do not agree you may cancel under Section 17. If a payment fails, we may suspend access to paid features until amounts due are paid.

9.7 Client charges, cancellations, and refunds. Any fees you charge your clients, and any cancellation, no-show, or refund policy you apply to them, are **set and operated by you** as part of your own practice. Faresay does not set, mandate, or adjudicate your client-facing pricing or cancellation terms. ⚠️ COUNSEL — confirm that client-facing consumer-protection obligations (e.g. Consumer Rights Act 2015 and Consumer Contracts Regulations 2013) rest with the Customer as the supplier of services to consumers, and that Faresay's terms make this clear.

10. Customer Responsibilities and Acceptable Use

10.1 You agree to: - (a) provide accurate registration, insurance, and account information and keep it up to date; - (b) use the Service only to run a lawful, properly registered practice; - (c) discharge your clinical, safeguarding, and data-protection responsibilities (Sections 6 and 11); and - (d) use the Service only for lawful purposes and in accordance with these Terms.

10.2 You agree **not** to: - (a) rely on the Service for any emergency or crisis (see Section 3); - (b) provide false information about your identity, registration, or entitlement to practise; - (c) use the Service if you are not, or cease to be, appropriately registered and insured; - (d) misuse client data or process it other than in accordance with your obligations as controller and the Data Processing Agreement; - (e) attempt to gain unauthorised access to the Service, interfere with its operation, introduce malicious code, or scrape or harvest data; - (f) resell, sublicense, or make the Service available to any third party except your own clients and authorised associates in the ordinary course of your practice; - (g) use the Service to infringe the rights of others or to store or transmit unlawful, defamatory, or infringing content; or - (h) use the Service in violation of any applicable law or regulation.

10.3 We may suspend or terminate access, and remove unlawful content, for breach of this Section.

11. Data Protection

11.1 Roles. In respect of your clients' personal data (including special-category health data) processed through the Service, **you are the controller and Faresay is the processor**, acting only on your documented instructions under the Data Processing Agreement (`data-processing-agreement.md`), which forms part of these Terms. In respect of your account, billing, and product-usage data, Faresay acts as an **independent controller**, as described in our Privacy Policy (`privacy-policy.md`).

11.2 Your controller duties. As controller of your clients' data, you are responsible for establishing a lawful basis and an Article 9 condition for processing special-category health data, providing privacy information to your clients, handling data-subject rights requests, and otherwise complying with the UK GDPR and the Data Protection Act 2018.

11.3 Faresay's processor duties. Faresay will process your clients' personal data only on your documented instructions and in accordance with the Data Processing Agreement, including the technical and organisational measures and the sub-processor terms described there.

11.4 Both parties will comply with the UK GDPR and the Data Protection Act 2018. ⚠️ COUNSEL — confirm the controller/processor split mirrors the Data Processing Agreement and Record of Processing Activities exactly.

12. Intellectual Property

12.1 The Service and all software, text, graphics, logos, trademarks, and other content provided by Faresay (excluding your content and your clients' content) are owned by or licensed to Faresay and protected by intellectual-property laws. "Faresay" and our logos are our trademarks.

12.2 We grant you a limited, non-exclusive, non-transferable, revocable licence to access and use the Service to run your practice in accordance with these Terms for the term of your subscription. No other rights are granted.

12.3 **Your content and client data.** You and your clients retain ownership of the content and data you and they submit. You grant Faresay a non-exclusive, worldwide, royalty-free licence to host, store, and process that content **solely as necessary to provide the Service and as instructed by you under the Data Processing Agreement**, and not for any independent purpose. This licence does not give Faresay any ownership of, or independent right to use, the clinical record, which remains yours. ⚠️ COUNSEL — confirm this licence is limited to provision of the Service and does not conflict with the controller/processor split or health-privacy law.

12.4 You must not use Faresay's intellectual property without our prior written consent.

13. Third-Party Providers and Links

13.1 The Service relies on and may integrate with third-party services (for example, our payment provider, video infrastructure, email/SMS delivery, and hosting). The current sub-processors are listed at `sub-processor-list.md`.

13.2 We are not responsible for the acts, omissions, content, products, or services of any third party beyond our own obligations under these Terms and the Data Processing Agreement, and your use of any third-party service is subject to that provider's terms. Inclusion of a provider or link is not an endorsement.

14. Disclaimers of Warranties

14.1 To the fullest extent permitted by law, the Service is provided "**as is**" and "**as available**", without warranties of any kind, whether express, implied, or statutory, including implied warranties of satisfactory quality, fitness for a particular purpose, and non-infringement, save that the Service will be supplied with reasonable care and skill.

14.2 We do not warrant that the Service will be uninterrupted, error-free, secure, or free of harmful components, or that any defect will be corrected, although we will use reasonable efforts to maintain availability and security.

14.3 **Faresay makes no warranty regarding the care you deliver.** Because Faresay does not provide clinical services, we make no representation or warranty as to the suitability, safety, quality, or outcome of any care you deliver using the Service. That care is your sole responsibility.

14.4 **Statutory rights.** ⚠️ COUNSEL — Even in a business-to-business contract, certain terms (for example, as to supply with reasonable care and skill) may be implied by law and cannot all be excluded. This Section must be qualified so it does not purport to exclude liabilities that cannot lawfully be excluded; counsel to add appropriate carve-out wording.

15. Limitation of Liability

15.1 To the fullest extent permitted by law, Faresay and its officers, directors, employees, and agents will not be liable for any indirect, incidental, special, consequential, or exemplary damages, or for any loss of profits, revenue, anticipated savings, data, goodwill, or other intangible losses, arising out of or relating to your use of (or inability to use) the Service.

15.2 To the fullest extent permitted by law, Faresay's total aggregate liability arising out of or relating to these Terms or the Service in any 12-month period will not exceed the **total subscription and commission fees you paid to Faresay in the 12 months** before the event giving rise to the claim. ⚠️ COUNSEL — confirm the cap and its enforceability in a B2B context.

15.3 **The care you provide.** Faresay is not liable for the care you provide, your clinical decisions, your compliance with professional or regulatory standards, or your discharge of your duties as data controller — all of which are your sole responsibility (Sections 6 and 11).

15.4 **Carve-outs.** ⚠️ COUNSEL — Nothing in these Terms limits or excludes liability that cannot lawfully be limited or excluded, including liability for death or personal injury caused by negligence, fraud or fraudulent misrepresentation, and any other liability that cannot be excluded under applicable law. Counsel to confirm the precise carve-outs.

16. Indemnification

16.1 To the fullest extent permitted by law, you agree to indemnify and hold harmless Faresay and its officers, directors, employees, and agents from and against any claims, liabilities, damages, losses, and expenses (including reasonable legal fees) arising out of or related to (a) your breach of these Terms, (b) your provision of care or operation of your practice, (c) your processing of personal data as controller, or (d) your violation of any law or the rights of any third party (including your clients). ⚠️ COUNSEL — confirm scope and enforceability in a B2B context.

17. Suspension, Termination, and Your Data

17.1 **Cancellation by you.** Paid plans renew automatically each month until cancelled. You may cancel at any time through your account; cancellation takes effect at the end of the then-current paid month, and you remain liable for fees accrued up to that point.

17.2 **Suspension or termination by us.** We may suspend or terminate your access, with or without notice, if (a) you breach these Terms, (b) you cease to meet the eligibility requirements in Section 4, (c) we are required to do so by law, (d) we reasonably believe your use poses a risk to others, to your clients' data, or to the Service, or (e) we discontinue the Service or a feature on reasonable notice.

17.3 **Export and deletion of your data.** On termination, you may export and keep your data, after which we delete or return it in accordance with the Data Processing Agreement. You remain responsible, as controller, for retaining the clinical record for the period your professional standards and applicable law require.

17.4 **Survival.** On termination, the licences granted to you end. Sections that by their nature should survive (including Sections 8, 11, 12, 14, 15, 16, 18, and the relevant terms of the Data Processing Agreement) survive termination.

17.5 ⚠️ COUNSEL / ⚠️ CLINICAL — Termination must not abruptly cut off your ability to retrieve records needed for continuity and retention. Coordinate with `clinical-governance-policy.md` and the Data Processing Agreement so that the Customer can export client and clinical data before deletion.

18. Changes to these Terms

18.1 We may update these Terms from time to time. If we make material changes, we will provide reasonable notice (for example, by email or an in-product notice) before they take effect.

18.2 Changes take effect on the date stated in the updated Terms. Your continued use of the Service after the effective date constitutes acceptance. If you do not agree, you may cancel under Section 17. ⚠️ COUNSEL — confirm change-of-terms mechanics for a B2B subscription.

19. Miscellaneous

19.1 **Entire agreement.** These Terms, together with the Privacy Policy, the Data Processing Agreement, and any other documents incorporated by reference, are the entire agreement between you and Faresay regarding the Service and supersede prior agreements on that subject. Where there is any inconsistency, the order of precedence is: the Data Processing Agreement (on data-processing matters), then these Terms, then any in-product or supplementary documents.

19.2 **Severability.** If any provision is held unenforceable, the remaining provisions remain in effect, and the unenforceable provision will be modified to the minimum extent necessary.

19.3 **No waiver.** Our failure to enforce any provision is not a waiver of it.

19.4 **Assignment.** You may not assign these Terms without our consent. We may assign these Terms in connection with a merger, acquisition, or sale of assets, subject to applicable law.

19.5 **No agency.** Nothing in these Terms creates any partnership, joint venture, agency, or employment relationship between you and Faresay. You act as an independent professional running your own practice.

19.6 **Force majeure.** We are not liable for any failure or delay caused by events beyond our reasonable control.

19.7 **Notices.** We may provide notices to you by email or through the Service. You may contact us as set out in Section 21.

19.8 **Third-party rights.** Except as expressly stated, a person who is not a party to these Terms has no right under the Contracts (Rights of Third Parties) Act 1999 to enforce any of its terms. ⚠️ COUNSEL — confirm.

20. Governing Law and Jurisdiction

20.1 These Terms, and any dispute or claim arising out of or in connection with them or their subject matter, are governed by the laws of **England & Wales**, and the courts of **England & Wales** have exclusive jurisdiction. ⚠️ COUNSEL — set governing law and venue for any later non-UK markets, and confirm whether any market-specific addendum is required.

20.2 **Informal resolution.** Before bringing a formal claim, you agree to contact us at [PLACEHOLDER: disputes contact] and attempt to resolve the dispute informally for at least [PLACEHOLDER: e.g. 30] days.

21. Contact

If you have questions about these Terms, contact us at:

- **Entity:** Faresay Ltd (company number 17302034)
- **Address:** 167-169 Great Portland Street, London W1W 5PF, England & Wales
- **Email:** [PLACEHOLDER: contact email, e.g. legal@ / enquiries@]

For urgent or emergency situations, do **not** contact us — see Section 3.

End of draft. ⚠ This document is a first draft for legal counsel review and is not legal advice. Counsel must finalise before use.

Therapist Agreement

⚠️ DRAFT v0.1 — for legal counsel review. NOT legal advice. Counsel must finalise before use. Last updated: 26 June 2026

Faresay Therapist Agreement

This Therapist Agreement (the "**Agreement**") is entered into between Faresay and the independent mental-health professional or practice that subscribes to the Faresay platform (the "**Therapist**"). It governs the Therapist's subscription to, and use of, the Faresay practice-management software.

By clicking to accept, signing, creating an account, or using the Faresay software, the Therapist agrees to be bound by this Agreement.

1. Parties and Definitions

1.1 Parties

1.1.1 "**Faresay**" means Faresay Ltd, a private limited company registered in England and Wales under company number 17302034 whose registered office is at 167-169 Great Portland Street, London W1W 5PF (also referred to as "**we**", "**us**", "**our**", and the "**Provider**").

⚠️ COUNSEL: Confirm the correct contracting entity before execution. The product is UK-first B2B SaaS; if a separate billing or group entity is introduced, confirm which entity contracts with the Therapist and is named here.

1.1.2 "**Therapist**" means the individual licensed or registered mental-health professional (or the practice) identified in the onboarding registration, who is subscribing in the course of their own business — as a sole trader, partnership, or company — and not as a consumer (also referred to as "**you**", "**your**", and the "**Customer**").

1.1.3 Faresay and the Therapist are each a "**Party**" and together the "**Parties**".

1.2 Definitions

In this Agreement:

- "**Client**" means an individual to whom the Therapist provides Clinical Services and whom the Therapist manages using the Software. Clients are the Therapist's own clients, not Faresay's.
- "**Clinical Record**" means the clinical notes, assessments, treatment plans, and other professional records the Therapist creates in connection with the care of a Client. The Therapist owns and holds the Clinical Record; Faresay does not.
- "**Clinical Services**" means the regulated mental-health / psychotherapy / counselling services the Therapist provides to their own Clients.
- "**Faresay Policies**" means the policies referenced in clause 17, including the Clinical Governance Policy (`clinical-governance-policy.md`), the Crisis/Safeguarding Policy (`crisis-safeguarding-policy.md`), the Privacy Policy (`privacy-policy.md`), the Security & Data Protection Policy (`security-data-protection-`

policy.md), the Data Processing Agreement (data-processing-agreement.md), and the Terms of Service (terms-of-service.md), each as updated from time to time.

- **"Processing Commission"** means the card-processing commission described in clause 5.3.
- **"Software"** or **"Platform"** means the Faresay website, applications, software, and related practice-management technology that the Therapist subscribes to, including tools for client management, scheduling, secure video sessions, reminders, and card payment collection.
- **"Subscription"** means the Therapist's paid subscription to the Software described in clause 5.
- **"Subscription Fee"** means the recurring fee payable for the Subscription, as described in clause 5.2.
- **"Territory"** means the United Kingdom.

⚠ COUNSEL: This Agreement is drafted for a UK-first launch. Any later expansion (e.g. to other nations or jurisdictions) will require jurisdiction-specific review and likely a separate addendum; do not assume this UK form ports unchanged.

2. Nature of the Relationship

2.1 Faresay is practice-management software. Faresay provides software-as-a-service that an independent professional subscribes to in order to run their own private practice — managing their own Clients, scheduling appointments, holding secure video sessions, sending reminders, taking card payments, and keeping practice records. Faresay is **software**, not a care provider, and **not** a marketplace, directory, matching service, referral service, or client-acquisition service. Faresay does not introduce, source, or supply Clients to the Therapist.

2.2 The Therapist brings and owns their own Clients. The Therapist runs their own practice and brings their own Clients to the Software. The professional relationship between the Therapist and each Client is exclusively the Therapist's. The Therapist exercises independent professional clinical judgement and is solely responsible for all clinical decisions. Faresay does not direct, control, or interfere with the Therapist's clinical judgement or the content of Clinical Services, and is not a party to the clinical relationship.

2.3 The Therapist owns and holds the Clinical Record. The Therapist is the owner and custodian of the Clinical Record, and is responsible for creating, maintaining, retaining, and securing it in accordance with applicable law and professional obligations. Faresay does not hold the Clinical Record. Faresay processes Client data only as a processor on the Therapist's instructions, as described in clause 9, the Data Processing Agreement, and the Privacy Policy.

2.4 Independent contractor; no employment or agency. The Therapist is an independent professional and not an employee, worker, partner, joint venturer, or agent of Faresay, and Faresay is not an employee or agent of the Therapist. Nothing in this Agreement creates an employment, agency, or partnership relationship. The Therapist is responsible for their own working methods, scheduling, fees, and the manner in which Clinical Services are delivered, consistent with applicable professional standards.

2.5 Characterisation of Faresay's fees. The Subscription Fee is a **fee for a software-as-a-service product** (the practice-management tools described in clause 2.1). The Processing Commission is a **fee for the card-payment-processing facility within the Software**. Neither is a division, split, share, or cut of the Therapist's professional or clinical fee, nor a marketplace fee, referral fee, or consideration for the introduction of Clients. The Therapist sets their own fees and keeps them, less only the underlying payment-processor fees and the Processing Commission.

⚠ COUNSEL: Confirm that the subscription-plus-processing-commission model, the in-product fee disclosures, and this software-fee characterisation are correctly described for a UK B2B SaaS offering, and that the Processing

Commission is clearly disclosed to the Therapist before they enable payments. Confirm the consumer/business boundary (clause 1.1.2) and that the Therapist contracts as a business, not a consumer.

3. Eligibility and Credentialing

3.1 Registration. The Therapist must hold, and maintain at all times, a valid, current, and unrestricted licence, registration, or accreditation to provide the relevant Clinical Services in the United Kingdom — for example current membership of an appropriate professional body or PSA-accredited register (such as BACP, UKCP, NCPS, BPS, or, where the profession is statutorily regulated such as HCPC, the relevant statutory registration) appropriate to the profession.

⚠ COUNSEL: Confirm the acceptable registers/bodies and any minimum-standing requirements for a UK launch, and how Faresay evidences and records eligibility at onboarding. See `clinical-governance-policy.md`.

3.2 Insurance. The Therapist must hold and maintain their own professional indemnity / professional liability insurance meeting the requirements of clause 13 throughout the term.

3.3 Identity and credential checks. The Therapist consents to, and must satisfactorily complete, identity verification and credential verification as Faresay reasonably requires at onboarding, and (where appropriate to the Therapist's practice) any further checks reasonably required.

3.4 Ongoing monitoring. The Therapist authorises Faresay to verify and to monitor on an ongoing basis the status of the Therapist's registrations and insurance, and to use third-party verification services for this purpose. The Therapist must notify Faresay **within [PLACEHOLDER: notice period, e.g. 3 business days]** of any change, lapse, suspension, condition, investigation, complaint, restriction, or revocation affecting any registration, insurance, or fitness to practise.

3.5 Accuracy. The Therapist must ensure all registration information and account/profile content remains accurate, current, and not misleading.

4. Software Provided to the Therapist

Subject to this Agreement and an active Subscription, Faresay provides the Therapist with access to:

4.1 client-management tools for the Therapist's own Client list; 4.2 scheduling, booking, and calendar tools; 4.3 secure messaging and a secure video-session environment, or integration with one; 4.4 appointment reminders; 4.5 card-payment collection from the Therapist's Clients (via the payment processor in clause 5.4), with payouts going to the Therapist; 4.6 administrative tooling to support light record-keeping, noting the Therapist remains the owner and holder of the Clinical Record per clause 2.3 (the Software stores light notes and, where applicable, links to documents the Therapist holds in their own storage); and 4.7 such other features as Faresay may make available from time to time.

Faresay provides the Software on an "as available" basis and may modify, add, or remove features, subject to clause 11.

5. Subscription, Fees and Payment Flow

5.1 Subscription. Access to the Software is provided on a **monthly Subscription** basis. The Subscription renews automatically each month until cancelled in accordance with clause 16.

5.2 Subscription Fee. The Therapist pays a recurring **Subscription Fee**, billed monthly in advance, for access to the Software. The applicable plan and price are [PLACEHOLDER: plan tiers and prices] and are disclosed in-product before the Therapist subscribes.

5.3 Processing Commission. Where the Therapist uses the Software to take card payments from their Clients, Faresay charges a **small Processing Commission** on each card payment processed, in addition to the Subscription Fee and the underlying payment-processor fees. The Processing Commission is a fee for the payment-processing facility within the Software and, as stated in clause 2.5, is **not** a marketplace fee, referral fee, or any share or split of the Therapist's professional fee. The Processing Commission rate is [PLACEHOLDER: commission rate] and is disclosed in-product before the Therapist enables payments.

5.4 Payment processor; payout. Card payments from Clients are collected and paid out to the Therapist's nominated account through Faresay's payment processor, [PLACEHOLDER: payment processor, e.g. Stripe]. The Therapist sets and owns their own Client fees and receives those fees less only the payment-processor fees and the Processing Commission. The Therapist must complete the processor's onboarding (including any required identity/KYC steps) and is subject to the processor's terms.

5.5 Refunds, cancellations, and chargebacks. Refunds, cancellations, and chargebacks on Client payments are a matter between the Therapist and their Client and are handled in accordance with the Terms of Service and the Therapist's own cancellation policy. Where a Client refund, reversal, or chargeback occurs, the Therapist remains responsible for the refunded amount and any associated processor fees, which Faresay (or the processor) may deduct or recover from current or future payouts or charge to the Therapist. ⚠️ **COUNSEL:** confirm allocation of chargeback liability and processor fee pass-through for a software/processing model.

5.6 Taxes. The Therapist is solely responsible for determining, reporting, and paying all taxes (including income tax, National Insurance, and VAT where applicable) arising from the Therapist's income and practice. Faresay does not withhold tax. Faresay invoices the Subscription Fee and Processing Commission and accounts for any VAT chargeable on those Faresay fees. The Therapist must provide any tax information reasonably required.

5.7 No charge tied to Clinical Services. Faresay's charges are the Subscription Fee and the Processing Commission only. Faresay does not charge for, take a percentage of, or otherwise share in the Therapist's Clinical Services or session fees.

6. Therapist Obligations

The Therapist must at all times:

6.1 Standard of care. Provide Clinical Services with the reasonable skill, care, and diligence expected of a competent professional in the relevant discipline, and in accordance with applicable professional, ethical, and regulatory standards.

6.2 Scope of practice and competence. Practise only within the scope of the Therapist's registration, training, and competence, and decline or refer where a Client's needs fall outside that scope (including where in-person or higher-acuity care is clinically indicated).

6.3 Telehealth compliance. Comply with all applicable telehealth / remote-consultation laws, professional guidance, and technical-safety requirements when delivering Clinical Services remotely, including verifying the Client's identity at each session as required.

6.4 Informed consent. Obtain and document appropriate informed consent for assessment and treatment, including consent to remote delivery, before providing Clinical Services.

6.5 Confidentiality. Maintain the confidentiality of Client information in accordance with professional duties of confidence, this Agreement, and applicable data-protection law (clause 9), disclosing only as permitted or required by law or to protect against serious harm.

6.6 Mandatory reporting and safeguarding. Comply with all safeguarding, mandatory-reporting, and duty-to-warn obligations applicable in the United Kingdom.

6.7 Crisis and safeguarding. Adhere to Faresay's Crisis/Safeguarding Policy (`crisis-safeguarding-policy.md`) and Clinical Governance Policy (`clinical-governance-policy.md`), each incorporated into this Agreement by reference, including escalation and risk-management procedures. Crisis handling, risk assessment, and safeguarding rest with the Therapist as the clinician; the Software is for scheduled, non-urgent care and is not a crisis or emergency service.

6.8 Record-keeping. Create, hold, and retain Clinical Records in their own keeping in accordance with applicable law and professional standards for the required retention period, and make them available to the Client (and, where lawfully required, to a continuing clinician) as required.

6.9 Conduct. Behave professionally toward Clients and Faresay staff, keep account and profile content accurate, and not make any false, misleading, or guaranteed-outcome claims.

6.10 Cooperation. Cooperate with Faresay's reasonable account-security, compliance, and support processes.

7. The Client Relationship

7.1 Clients are the Therapist's. The Software is a tool the Therapist uses to manage their own practice. All Clients the Therapist manages using the Software are the Therapist's own Clients. Faresay does not own, introduce, or have any claim to the Client relationship, and acquires no interest in the Therapist's Clients by reason of the Therapist's use of the Software.

7.2 No restriction on the Therapist's practice. Faresay does not restrict the Therapist from practising, taking on Clients, using other tools, or continuing to treat any Client outside the Software. The Therapist is free to use or stop using the Software at any time in accordance with clause 16, and may export their data under clause 16.5 and the Data Processing Agreement.

7.3 Continuity of care. The Therapist remains responsible at all times for continuity of care and for not abandoning a Client, including on any cancellation or termination of the Subscription (see clause 15).

 **COUNSEL:** Confirm there are no residual marketplace-style restrictions (e.g. non-solicitation or non-circumvention) anywhere in the Therapist-facing terms, consistent with the software-subscription model in which Faresay does not supply Clients.

8. (Reserved)

This clause number is intentionally reserved.

9. Data Protection and Confidentiality

9.1 Roles. With respect to Client personal data managed through the Software, the **Therapist is the controller** (as the clinician who owns the Client relationship and holds the Clinical Record) and **Faresay is the processor**, acting only on the Therapist's documented instructions under the Data Processing Agreement (`data-processing-agreement.md`). Faresay is an **independent controller** only for the limited account-administration, billing, subscription, support, and security data described in the Privacy Policy (`privacy-policy.md`) and the Security & Data Protection Policy (`security-data-protection-policy.md`).

⚠️ COUNSEL / DPO: Confirm the controller/processor split is correctly mapped per processing activity and that the UK GDPR Article 28 arrangements in the Data Processing Agreement are in place before launch. The split must match the Privacy Policy and DPA, under which the Therapist is the controller of Client/clinical data and Faresay is the processor.

9.2 UK GDPR. Both Parties must comply with the UK GDPR and the Data Protection Act 2018. Client clinical data is special-category (health) data: as controller, the Therapist is responsible for identifying an appropriate lawful basis and Article 9 condition (for example Article 9(2)(h) provision of health/social-care services, or Article 9(2)(a) explicit consent), providing privacy information to Clients, and responding to data-subject rights requests. Faresay assists the Therapist with these obligations as set out in the Data Processing Agreement.

9.3 Client requests. Where a Client contacts Faresay directly to exercise rights over their data, Faresay will refer them to the Therapist as controller and assist the Therapist as required under the Data Processing Agreement. Faresay does not decide why or how a Client's data is used and does not hold the Clinical Record.

⚠️ COUNSEL: Confirm the security-incident, sub-processor, and international-transfer mechanics in the Data Processing Agreement (e.g. Stripe, and other sub-processors) and the Security & Data Protection Policy are consistent with this Agreement. See `data-processing-agreement.md` and `sub-processor-list.md`.

9.4 Security. The Therapist must comply with the Security & Data Protection Policy, use only Faresay-approved or otherwise compliant tools to handle Client data, and report any personal-data breach within the Therapist's control to Faresay without undue delay and in any event within [PLACEHOLDER: e.g. 24 hours] of becoming aware of it. Faresay's own breach-notification obligations to the Therapist are set out in the Data Processing Agreement.

9.5 Confidential information. Each Party must keep confidential the other's non-public business information and use it only to perform this Agreement. This clause 9.5 does not limit the Therapist's separate professional duty of clinical confidentiality owed to Clients.

10. Intellectual Property and Licence

10.1 Faresay IP. Faresay (and its licensors) own all rights in the Software, the Faresay name, logos, and marks, and all Faresay-created content. No rights are granted except as expressly set out here.

10.2 Licence to the Therapist. Faresay grants the Therapist, for the term of an active Subscription, a non-exclusive, non-transferable, revocable licence to access and use the Software to run their own practice in accordance with this Agreement.

10.3 Therapist content. The Therapist retains ownership of content and data they supply (such as profile text, Client records, and notes) and grants Faresay a non-exclusive, royalty-free licence to host, store, and process that content solely to provide and support the Software in accordance with this Agreement and the Data Processing Agreement. Faresay does not use Client data for its own purposes, marketing, or to train AI models.

10.4 Clinical Record. For the avoidance of doubt, this clause 10 does not affect the Therapist's ownership and holding of the Clinical Record under clause 2.3.

11. Representations and Warranties

11.1 The Therapist represents and warrants that, on an ongoing basis:

11.1.1 they hold all registrations required to provide the Clinical Services in the United Kingdom (clause 3); 11.1.2 they are not subject to any unspent disqualification, suspension, exclusion, or restriction that would prevent them

from providing Clinical Services; 11.1.3 they hold the insurance required by clause 13; 11.1.4 all information they provide to Faresay is accurate and not misleading; and 11.1.5 they will comply with all applicable laws, professional standards, and Faresay Policies.

11.2 Except as expressly stated, the Software is provided "as is" and "as available" and Faresay disclaims all other warranties to the maximum extent permitted by law. Because Faresay does not provide care, Faresay makes no warranty as to the suitability, safety, quality, or outcome of any Clinical Services the Therapist delivers using the Software. Nothing in this clause excludes Faresay's obligation to supply the Software with reasonable care and skill.

12. Indemnification

12.1 The Therapist will indemnify and hold harmless Faresay and its affiliates, and their officers and staff, from and against losses, liabilities, damages, claims, and reasonable costs (including legal fees) arising out of or in connection with:

12.1.1 the Therapist's provision of (or failure to provide) Clinical Services, including any clinical negligence, malpractice, or breach of professional duty; 12.1.2 the Therapist's breach of this Agreement, the Faresay Policies, or applicable law; 12.1.3 any claim that the Therapist lacked the required registration or competence; and 12.1.4 the Therapist's breach of its data-protection obligations as controller of Client data, or of its confidentiality obligations.

12.2 Faresay will indemnify the Therapist for losses arising from Faresay's breach of this Agreement or its negligence in providing the Software, subject to clause 14 (limitation of liability).

 COUNSEL: Mutual scope, caps, and carve-outs, and the interaction with insurance and with the limitation of liability, must be reviewed. Certain liabilities (e.g. the Therapist's clinical negligence to a Client) rest with the Therapist and must be backed by the Therapist's insurance.

13. Insurance Requirements

13.1 The Therapist must, at their own cost, hold and maintain professional indemnity / professional liability insurance with a reputable insurer, with cover of at least [PLACEHOLDER: minimum cover amount and basis, e.g. per-claim / aggregate] appropriate to the Clinical Services they provide.

13.2 The Therapist must provide evidence of cover on onboarding and on request, and must notify Faresay promptly if cover lapses, is reduced, or is cancelled.

13.3 Cover must remain in force during the term and, where the policy is claims-made, for [PLACEHOLDER: run-off / tail period] after termination.

14. Limitation of Liability

14.1 Nothing in this Agreement limits or excludes liability that cannot lawfully be limited or excluded, including liability for death or personal injury caused by negligence, for fraud, or any other liability that cannot be limited under applicable law.

14.2 Subject to clause 14.1, Faresay is not liable for indirect or consequential loss, loss of profit, loss of goodwill, or loss of anticipated savings.

14.3 Subject to clauses 14.1 and 14.2, Faresay's total aggregate liability arising out of or in connection with this Agreement is limited to [PLACEHOLDER: cap, e.g. the total Subscription Fees and Processing Commission paid by the Therapist to Faresay in the [12] months before the claim].

14.4 For the avoidance of doubt, Faresay is not responsible for the Clinical Services, for the Therapist's clinical decisions, or for the Therapist's discharge of its duties as controller of Client data — all of which are the Therapist's sole responsibility.

⚠️ COUNSEL: Caps and exclusions must be reviewed for enforceability under UK law (e.g. Unfair Contract Terms Act 1977, and the B2B nature of the contract), and reconciled with the indemnity (clause 12) and insurance (clause 13).

15. Continuity of Care on Cancellation or Termination

15.1 The welfare of Clients takes priority on any cancellation, suspension, or termination. The Therapist must not abandon a Client and must comply with their professional continuity-of-care obligations.

15.2 On cancellation or termination, the Therapist remains responsible for managing an orderly transition of their own Clients, which may include:

15.2.1 completing or safely concluding in-progress care, or arranging an appropriate handover or referral; 15.2.2 retaining and making available to the Client (and any continuing clinician the Client chooses) the relevant Clinical Record as required by law and professional standards; and 15.2.3 continuing to use the Software during any wind-down period the Therapist requires, subject to an active Subscription, or exporting their data under clause 16.5.

15.3 Because Clients are the Therapist's own, continuity of care and Client communication rest with the Therapist; Faresay's role is limited to making the Software and the Therapist's data available as set out in clause 16.5 and the Data Processing Agreement.

⚠️ COUNSEL / ⚠️ CLINICAL: Continuity-of-care and record-retention mechanics must align with `clinical-governance-policy.md`, `crisis-safeguarding-policy.md`, and UK professional ethics rules.

16. Term, Suspension, and Termination

16.1 **Term.** This Agreement starts when the Therapist accepts it and continues for so long as the Therapist maintains a Subscription, until terminated under this clause 16.

16.2 **Cancellation by the Therapist.** The Therapist may cancel the Subscription at any time through their account. Cancellation takes effect at the end of the then-current paid month, and Subscription Fees already paid for the current month are non-refundable except as required by law.

16.3 **Termination for cause by Faresay.** Faresay may suspend access or terminate immediately on written notice if the Therapist:

16.3.1 loses, or has restricted, any required registration or insurance; 16.3.2 is subject to a regulatory, safeguarding, or fitness-to-practise concern that Faresay reasonably considers presents a risk to Clients; 16.3.3 commits a material breach of this Agreement or the Faresay Policies (and, if remediable, fails to remedy it within [PLACEHOLDER: cure period]); 16.3.4 fails to pay the Subscription Fee or any sum due when due; or 16.3.5 becomes insolvent or ceases to practise.

16.4 **Suspension.** Faresay may suspend the Therapist's access pending investigation of any matter under clause 16.3 where reasonably necessary to protect Clients or the integrity of the Software, acting in good faith.

16.5 Effect of termination; data export. On termination, the licence in clause 10.2 ends and the Therapist must stop using the Software and Faresay marks. The Therapist may export their data during the term and for a period after termination, after which Faresay returns or deletes it, in accordance with the Data Processing Agreement. The Therapist remains responsible, as controller, for retaining the Clinical Record for the period their professional standards require. Accrued payment rights, and clauses intended to survive (including clauses 2.3, 5.5–5.6, 7, 9, 11–15, 17, 19, and 20), survive termination.

17. Compliance with Faresay Policies

17.1 The Therapist must comply with all Faresay Policies, which are incorporated into this Agreement by reference, including:

- Clinical Governance Policy — `clinical-governance-policy.md`
- Crisis/Safeguarding Policy — `crisis-safeguarding-policy.md`
- Privacy Policy — `privacy-policy.md`
- Security & Data Protection Policy — `security-data-protection-policy.md`
- Data Processing Agreement — `data-processing-agreement.md`
- Terms of Service — `terms-of-service.md`

17.2 Faresay may update the Faresay Policies from time to time in line with clause 20.2. Where a Policy conflicts with this Agreement on a clinical-safety or legal-compliance matter, the more protective requirement applies; otherwise, on data-protection matters the Data Processing Agreement prevails, and on all other matters this Agreement prevails.

18. Dispute Resolution and Governing Law

18.1 The Parties will first attempt in good faith to resolve any dispute through discussion and, if needed, [PLACEHOLDER: escalation / mediation process].

18.2 This Agreement and any dispute arising out of it are governed by the laws of England and Wales, and the Parties submit to the exclusive jurisdiction of the courts of England and Wales.

 **COUNSEL:** Confirm governing law and venue for a UK launch (England and Wales as drafted), and whether any Scotland / Northern Ireland variation is needed if serving those nations. Confirm whether any arbitration provision is wanted for this B2B contract.

19. Notices

19.1 Notices under this Agreement must be in writing and sent to: (a) Faresay at [PLACEHOLDER: legal/notices email and address, e.g. legal@faresay.com]; and (b) the Therapist at the contact details in their account.

19.2 Routine operational communications may be given through the Software or by email and are deemed received [PLACEHOLDER: e.g. on the next business day].

20. Entire Agreement, Amendments, and General

20.1 **Entire agreement.** This Agreement and the documents it incorporates by reference are the entire agreement between the Parties on its subject matter and supersede prior discussions.

20.2 **Amendments.** Faresay may amend this Agreement or the Faresay Policies on [PLACEHOLDER: notice period] notice; continued use of the Software after the effective date constitutes acceptance. Material changes adverse to the Therapist will be notified, and the Therapist may cancel under clause 16.2 if they do not accept them. ⚠️
COUNSEL: confirm unilateral-amendment mechanics are enforceable for a UK B2B contract.

20.3 **Assignment.** The Therapist may not assign this Agreement without Faresay's consent. Faresay may assign to an affiliate or successor.

20.4 **No partnership/agency.** Nothing creates a partnership or agency (clause 2.4).

20.5 **Severance.** If any provision is unenforceable, the rest remains in effect and the provision is modified to the minimum extent necessary.

20.6 **Waiver.** A failure to enforce any right is not a waiver of it.

20.7 **Third-party rights.** Except as expressly stated, no third party has rights to enforce this Agreement [PLACEHOLDER: confirm Contracts (Rights of Third Parties) Act 1999 position].

End of draft. ⚠️ This document is a v0.1 first draft and must be reviewed and finalised by qualified UK legal counsel before use. It is not legal advice.

Data Processing Agreement (UK GDPR Art 28)

⚠️ DRAFT v0.1 — template for professional sign-off. NOT legal advice. A solicitor must finalise before reliance. Last updated: 25 June 2026

Faresay Data Processing Agreement

This Data Processing Agreement ("**DPA**") forms part of, and is incorporated into, the [Faresay Therapist Agreement](#) and the [Terms of Service](#) (together, the "**Agreement**") between Faresay and the therapist, practitioner or practice that subscribes to the Faresay platform (the "**Customer**").

It governs the processing of personal data carried out by Faresay on the Customer's behalf and is designed to satisfy **Article 28 of the UK GDPR** and the **Data Protection Act 2018**.

Roles. Under this DPA the **Customer is the Controller** of the clinical and client personal data they manage on the platform, and **Faresay is the Processor**, acting only on the Customer's documented instructions. Faresay is an independent Controller only for the limited account-administration, billing and security data described in Annex 1. This split reflects Faresay's product model: the therapist owns the client relationship and the clinical record; Faresay provides the software.

1. Definitions

Terms not defined here have the meaning given in the UK GDPR. In this DPA:

- "**UK GDPR**" means Regulation (EU) 2016/679 as it forms part of UK law by virtue of the European Union (Withdrawal) Act 2018, together with the Data Protection Act 2018.
- "**Controller**", "**Processor**", "**Data Subject**", "**Personal Data**", "**Special Category Data**", "**Processing**", "**Personal Data Breach**" have the meanings in the UK GDPR.
- "**Customer Personal Data**" means Personal Data that Faresay Processes on the Customer's behalf under the Agreement, as described in **Annex 1**.
- "**Sub-processor**" means any third party engaged by Faresay to Process Customer Personal Data. The current list is at **Annex 3** ([Sub-processor List](#)).
- "**TOMs**" means the technical and organisational measures at **Annex 2** ([Technical & Organisational Measures](#)).

2. Subject matter, duration, nature and purpose

Item	Detail
Subject matter	Provision of the Faresay practice-management platform to the Customer.
Duration	For the term of the Agreement, plus any post-termination period in Section 11.
Nature	Hosting, storage, transmission, scheduling, payment facilitation, video session hosting, messaging, reminders and related processing operations.

Item	Detail
Purpose	To enable the Customer to run their therapy practice (manage clients, book and deliver sessions, take payment, send reminders).
Categories of Data Subject	The Customer's clients; the Customer's own staff/associates where applicable.
Categories of Personal Data	Identity and contact details; appointment and scheduling data; payment metadata; messages; client-supplied intake responses; special-category health data (mental-health information).

The full detail is set out in **Annex 1**.

3. Processing only on documented instructions

3.1 Faresay shall Process Customer Personal Data **only on the documented instructions of the Customer**, including as set out in the Agreement, this DPA, and as given through the platform's features, unless required to do otherwise by UK law (in which case Faresay shall, where legally permitted, inform the Customer first).

3.2 Faresay shall **immediately inform** the Customer if, in its opinion, an instruction infringes the UK GDPR or other data-protection law.

3.3 Faresay shall not sell Customer Personal Data, nor use it for its own purposes, profiling, marketing, or to train AI models.

4. Confidentiality

Faresay shall ensure that persons authorised to Process Customer Personal Data are bound by an appropriate duty of confidentiality and Process the data only as instructed. Access is granted on a least-privilege, need-to-know basis (see Annex 2).

5. Security (Article 32)

5.1 Faresay shall implement and maintain the technical and organisational measures in **Annex 2**, appropriate to the risk, taking into account the special-category nature of the data.

5.2 Faresay may update the TOMs from time to time provided the level of protection is not materially reduced.

6. Sub-processors

6.1 The Customer **grants general authorisation** for Faresay to engage the Sub-processors listed in **Annex 3**.

6.2 Faresay shall impose data-protection obligations on each Sub-processor that are no less protective than those in this DPA, by written contract, and remains **fully liable** to the Customer for each Sub-processor's performance.

6.3 Faresay shall give the Customer **at least 14 days' notice** (by email and/or in-product) before adding or replacing a Sub-processor. The Customer may object on reasonable data-protection grounds; if the parties cannot resolve the objection, the Customer may terminate the affected service.

7. Data-subject rights

Taking into account the nature of the Processing, Faresay shall assist the Customer by appropriate technical and organisational measures, insofar as possible, to fulfil the Customer's obligation to respond to requests to exercise Data Subject rights (access, rectification, erasure, restriction, portability, objection). Where a Data Subject contacts Faresay directly about Customer Personal Data, Faresay shall, without undue delay, refer them to the Customer.

8. Personal Data Breach

8.1 Faresay shall notify the Customer **without undue delay, and in any event within 48 hours**, after becoming aware of a Personal Data Breach affecting Customer Personal Data.

8.2 The notification shall describe, to the extent known: the nature of the breach; categories and approximate number of Data Subjects and records affected; likely consequences; and measures taken or proposed. Faresay shall provide reasonable assistance to the Customer in meeting the Customer's own breach-notification duties to the ICO and Data Subjects (UK GDPR Arts 33–34).

9. DPIAs and prior consultation

Faresay shall provide the Customer with reasonable assistance with data-protection impact assessments (Art 35) and prior consultation with the ICO (Art 36), taking into account the nature of Processing and the information available to Faresay. A platform-level DPIA is maintained at [Data Protection Impact Assessment](#).

10. International transfers

10.1 Faresay primarily stores Customer Personal Data on infrastructure located in the **UK / EEA** (see Annex 3).

10.2 Where any transfer of Customer Personal Data to a third country occurs (for example via a Sub-processor), Faresay shall ensure an appropriate safeguard is in place — the **UK International Data Transfer Agreement (IDTA)**, the **UK Addendum to the EU Standard Contractual Clauses**, or an adequacy regulation — together with any supplementary measures required following a transfer risk assessment.

11. Return and deletion

11.1 On termination of the Agreement, Faresay shall, at the Customer's choice, **return or delete** all Customer Personal Data, and delete existing copies, unless UK law requires storage.

11.2 The Customer may **export their data at any time** during the term via the platform's export features. Faresay shall make Customer Personal Data available for export for **30 days** after termination, after which it shall be deleted within a further **90 days**, subject to routine backup-rotation cycles.

12. Audit

12.1 Faresay shall make available to the Customer all information reasonably necessary to demonstrate compliance with Article 28 and this DPA.

12.2 Faresay shall allow for and contribute to audits, including inspections, conducted by the Customer or an auditor it mandates, on reasonable prior notice (at least 30 days), no more than once per 12-month period (save where required by a supervisory authority or following a breach), subject to confidentiality. Faresay may satisfy audit requests by providing its security documentation, the TOMs, and responses to a reasonable security questionnaire.

13. Liability and precedence

13.1 The liability provisions of the Agreement apply to this DPA.

13.2 In the event of conflict between this DPA and the rest of the Agreement on data-protection matters, **this DPA prevails.**

14. Governing law

This DPA is governed by the laws of **England and Wales** and subject to the exclusive jurisdiction of its courts.
England and Wales (solicitor to confirm any Scotland/NI variation if serving those nations)

Annex 1 — Details of Processing

- **Controller:** the Customer (therapist / practice).
- **Processor:** Faresay Ltd, a company registered in England and Wales (company number 17302034), registered office 167-169 Great Portland Street, London W1W 5PF.
- **Subject matter, duration, nature, purpose:** as Section 2.
- **Categories of Data Subject:** the Customer's clients; the Customer's staff/associates where applicable.
- **Categories of Personal Data:** name, email, phone; date of birth (optional); appointment dates/times; session/booking history; payment metadata (card details are handled by Stripe — Faresay does not store card numbers); messages between Customer and client; client intake/consent responses; **special-category health data** (mental-health information the Customer or client records).
- **Special-category data:** Yes — health data. Processed because it is necessary for the provision of health/social-care services by a health professional under a duty of confidentiality (UK GDPR Art 9(2)(h)) and/or with explicit consent (Art 9(2)(a)), as determined by the Controller.
- **Faresay-as-Controller data (outside this DPA):** the Customer's own account/login credentials, subscription and billing records, platform audit logs, aggregate/cookieless usage analytics, and security telemetry — Processed by Faresay as an independent Controller under the [Privacy Policy](#).

Annex 2 — Technical & Organisational Measures

See [Technical & Organisational Measures](#).

Annex 3 — Sub-processors

See [Sub-processor List](#).

This DPA is a draft template prepared to support a UK launch. It must be reviewed and finalised by a qualified data-protection solicitor before it is offered to customers.

Technical & Organisational Measures (Annex 2)

⚠️ DRAFT v0.1 — template for professional sign-off. NOT legal advice. Some controls describe target state; each is flagged. Last updated: 25 June 2026

Faresay Technical & Organisational Measures (TOMs)

This document is **Annex 2** to the [Data Processing Agreement](#). It describes the technical and organisational security measures Faresay maintains under **Article 32 of the UK GDPR**, appropriate to the risk of processing **special-category (mental-health) data**.

Faresay is early-stage and bootstrapped. Controls marked **[target]** are planned or partially implemented and being matured; all others are in place today. This annex is reviewed at least annually and on material change.

1. Pseudonymisation and encryption

- **Encryption in transit:** all traffic to and from the platform is served over **TLS 1.2+ (HTTPS)**. Video sessions use the encrypted transport provided by the video sub-processor.
- **Encryption at rest:** the primary database (Neon Postgres) and object/file storage encrypt data at rest using the provider's managed encryption (AES-256).
- **Secrets:** API keys and credentials are held as environment secrets in the hosting platform, never committed to source control.
- **Card data:** never stored by Faresay — card details are tokenised by **Stripe** (PCI-DSS Level 1 provider).

2. Confidentiality

- **Access control:** access to production data is least-privilege and limited to personnel who require it. Authentication to the platform is handled by **Clerk** with support for strong passwords and multi-factor authentication.
- **Therapist–client isolation:** application-level authorisation ensures a Customer can access only their own clients, sessions and records.
- **Staff confidentiality:** all personnel with data access are bound by confidentiality obligations.
- **Background checks [target]:** appropriate vetting for any future staff with production access.

3. Integrity

- **Audit logging:** security-relevant events and administrative actions are logged.
- **Change control:** code changes go through version control (Git) and review before deploying to production; infrastructure is managed as code where practical.
- **Input validation:** server-side validation and rate-limiting on public endpoints (e.g. self-booking) to mitigate abuse and injection.

- **Dependency hygiene [target]:** automated dependency and vulnerability scanning.

4. Availability and resilience

- **Managed infrastructure:** hosting (Vercel) and database (Neon) are managed, redundant cloud services with provider-level high availability.
- **Backups:** the database provider maintains automated backups and point-in-time recovery.
- **Restore testing [target]:** periodic test of backup restoration.

5. Resilience and recovery (Art 32(1)(c))

- **Incident response:** a documented process to detect, triage, contain and notify on a Personal Data Breach, including the **48-hour** Customer-notification commitment in the DPA.
- **Business continuity [target]:** documented recovery objectives (RPO/RTO) as the service scales.

6. Testing and evaluation (Art 32(1)(d))

- **Review cadence:** TOMs and security posture reviewed at least annually.
- **Penetration testing [target]:** independent testing prior to, or shortly after, scaled launch.

7. Data minimisation and retention

- Only data necessary to run the practice is collected. Health data is recorded at the Customer's discretion.
- Retention and deletion follow the DPA (export window + deletion timeline) and the [Privacy Policy](#).

8. Sub-processor security

- Sub-processors are selected for their security posture and bound by contractual data-protection obligations no less protective than the DPA. See [Sub-processor List](#).

9. Organisational measures

- **Data protection by design and default** is applied to new features (e.g. cookieless analytics; SMS used only for appointment reminders).
- **Policies:** this annex is supported by the [Security & Data Protection Policy](#), [Clinical Governance Policy](#) and [Crisis & Safeguarding Policy](#).
- **ICO registration:** Faresay is registered (or registering) with the ICO and pays the data-protection fee. *ICO registration applied for 26 June 2026 — registration number to follow.*

Draft prepared to support a UK launch. To be validated by a security specialist and data-protection solicitor before reliance.

Sub-processor List (Annex 3)

⚠️ DRAFT v0.1 — template for professional sign-off. NOT legal advice. Provider regions/terms must be verified against each provider's current DPA. Last updated: 25 June 2026

Faresay Sub-processor List

This document is **Annex 3** to the [Data Processing Agreement](#). It lists the third parties Faresay engages to Process Customer Personal Data, the purpose, the data involved, the hosting location, and the international-transfer safeguard where data leaves the UK/EEA.

Customers receive **at least 14 days' notice** before a new sub-processor is added (DPA Section 6). To subscribe to change notices: email to the address on the Customer's account; changes are also posted in-product.

Verify before reliance: each provider's processing region and current data-processing terms must be confirmed at finalisation — providers change defaults and sub-processor terms over time. Where a US transfer applies, the safeguard is the **UK IDTA** or the **UK Addendum to the EU SCCs**, supported by a transfer risk assessment.

Infrastructure sub-processors

Sub-processor	Purpose	Data processed	Location	Transfer safeguard
Neon	Primary database hosting (Postgres)	All structured platform data	AWS London (eu-west-2) — UK/EEA <i>[verify]</i>	UK/EEA — none required
Vercel	Application hosting & serverless compute	Transient request data in processing	Global edge; US parent	UK Addendum / IDTA

Application sub-processors

Sub-processor	Purpose	Data processed	Location	Transfer safeguard
Clerk	Authentication & identity	Account credentials, name, email	US <i>[verify]</i>	UK Addendum / IDTA
Stripe	Payment processing & subscriptions	Payment metadata; card data (tokenised by Stripe — not stored by Faresay)	Stripe Payments Europe (Ireland) + US	SCCs / UK Addendum
Daily.co	Video session hosting	Live audio/video session media (not stored by Faresay); display names	US <i>[verify]</i>	UK Addendum / IDTA

Sub-processor	Purpose	Data processed	Location	Transfer safeguard
Resend	Transactional email (bookings, reminders, receipts)	Recipient email; message content (appointment details)	US <i>[verify]</i>	UK Addendum / IDTA
Twilio	SMS appointment reminders	Client mobile number; reminder text	US / Ireland <i>[verify]</i>	SCCs / UK Addendum

Analytics sub-processors

Sub-processor	Purpose	Data processed	Location	Transfer safeguard
Plausible Analytics	Cookieless web analytics	Aggregate usage only — no cookies, no personal data, no cross-site tracking	EU (EU-hosted) <i>[verify]</i>	UK/EEA — none required

Notes

- **Card details** are never stored by Faresay — they are entered directly into Stripe's PCI-DSS-compliant systems and returned to Faresay only as a token.
- **Health data** is held in the primary database (Neon) and transmitted through the application and email/SMS sub-processors only to the extent the Customer's chosen features require (e.g. an appointment reminder contains the practitioner name and time, not clinical content).
- **Plausible** is used specifically because it is cookieless and processes no personal data, minimising Faresay's analytics footprint.

Draft prepared to support a UK launch. Regions and transfer mechanisms to be confirmed by a data-protection solicitor before reliance.

Record of Processing Activities (Art 30)

 DRAFT v0.1 — template for professional sign-off. NOT legal advice. To be maintained as a living record. Last updated: 25 June 2026

Faresay Record of Processing Activities (ROPA)

This is Faresay's record under **Article 30 of the UK GDPR**. Faresay holds two roles:

- **As Processor** (Art 30(2)) — for the client/clinical data it processes on behalf of Customers (therapists).
- **As Controller** (Art 30(1)) — for its own account-administration, billing, security and analytics data.

It is a living document, reviewed at least annually and on material change.

Controller / Processor identity

- **Organisation:** Faresay Ltd (company number 17302034).
- **Registered address:** 167-169 Great Portland Street, London W1W 5PF.
- **ICO registration number:** applied for 26 June 2026 — registration number to follow.
- **Data protection contact:** Joel Harvey, Founder — support@faresay.com.

Part A — Processing as PROCESSOR (on behalf of Customers)

Field	Detail
Controllers on whose behalf we act	Faresay's subscribing therapists / practices ("Customers").
Categories of processing	Hosting, storage, transmission, scheduling, payment facilitation, video hosting, messaging, email/SMS reminders, data export and deletion.
Categories of Data Subject	Customers' clients; Customers' staff/associates.
Categories of Personal Data	Identity & contact (name, email, phone); date of birth (optional); appointment/scheduling data; payment metadata; messages; intake/consent responses; special-category health data .
Special-category data	Health (mental-health) data — Art 9(2)(h)/(a) as determined by the Controller.
Transfers to third countries	Via sub-processors only; safeguarded by UK IDTA / UK Addendum to EU SCCs. See Sub-processor List .
Retention	For the term; export available 30 days post-termination; deletion within a further 90 days (subject to backup rotation).

Field	Detail
Security measures	See Technical & Organisational Measures .

Part B — Processing as CONTROLLER (Faresay's own purposes)

Purpose	Data subjects	Personal data	Lawful basis	Retention
Account administration	Customers (therapists)	Name, email, login credentials, practice details	Contract (Art 6(1)(b))	Life of account + 12 months
Subscription & billing	Customers	Subscription status, payment metadata, invoices	Contract; legal obligation (tax records)	6 years (UK tax)
Platform security & audit logs	Customers; clients	IP, device, timestamps, security events	Legitimate interests (Art 6(1)(f)) — security	12 months
Service emails (transactional)	Customers	Email, account events	Contract	Life of account
Product analytics	Website visitors	Aggregate, cookieless — no personal data (Plausible)	Legitimate interests	N/A (no personal data)
Marketing / newsletter <i>(if used)</i>	Subscribers	Email	Consent (Art 6(1)(a))	Until unsubscribe
Support & dispute handling	Customers; clients	Correspondence	Legitimate interests; legal claims	to be confirmed

Transfers summary

Primary storage is **UK/EEA** (Neon, London). Where a sub-processor processes data outside the UK/EEA, transfers are safeguarded by the **UK IDTA** or **UK Addendum to the EU SCCs**, supported by a transfer risk assessment. Full mapping: [Sub-processor List](#).

Lawful basis for special-category data

For client health data processed **as Processor**, the lawful basis is determined by the Customer (Controller) — typically Art 9(2)(h) (provision of health/social-care by a professional under a confidentiality duty) and/or Art 9(2)(a) (explicit consent). Faresay does not determine the purposes of the clinical processing.

Draft prepared to support a UK launch. To be completed (placeholders) and validated by a data-protection solicitor.

Data Protection Impact Assessment (Art 35)

⚠️ DRAFT v0.1 — template for professional sign-off. NOT legal advice. Risk scoring to be validated by the DPO/solicitor. Last updated: 25 June 2026

Faresay Data Protection Impact Assessment (DPIA)

Prepared under **Article 35 of the UK GDPR**, following the ICO's DPIA methodology. A DPIA is **required** here because the processing involves **large-scale processing of special-category (health) data** using new technology — two of the ICO's screening triggers.

Assessment owner	Joel Harvey, Founder
DPO / data-protection lead	Joel Harvey, Founder (no statutory DPO required at current scale)
Date started / completed	25 June 2026
Review date	At least annually, or on material change

Step 1 — Identify the need for a DPIA

Faresay is a practice-management platform that therapists subscribe to in order to run their practice: managing clients, booking and delivering (video) sessions, taking card payment, and sending email/SMS reminders. Faresay acts as **Processor** for the clinical/client data; the therapist is the **Controller**.

A DPIA is required because the processing involves: (a) **special-category health data**; (b) at **scale** as the platform grows; (c) **vulnerable data subjects** (people seeking mental-health support); and (d) **new technological solutions** (video, automated reminders).

Step 2 — Describe the processing

Nature: collection, storage, transmission, display, payment facilitation, video hosting, messaging, automated reminders, export and deletion.

Scope: identity/contact data, scheduling data, payment metadata, messages, intake/consent responses, and **mental-health information**. Data subjects are clients of subscribing therapists (and the therapists themselves). Volume grows with adoption.

Context: clients are often vulnerable; there is a clear power/confidentiality dynamic in therapy. Data is among the most sensitive categories that exist. Faresay is UK-first.

Purposes: to let independent therapists deliver and administer care efficiently. Faresay's purpose as Processor is limited to providing the software on the Controller's instructions.

Data flows: client → therapist's practice on Faresay → stored in UK/EEA database (Neon); payments via Stripe; video via Daily.co; email via Resend; SMS via Twilio; cookieless analytics via Plausible. See [Sub-processor List](#).

Step 3 — Consultation

- **Data subjects / therapists:** To be recorded during design-partner onboarding.
- **Internal:** founder/engineering; DPO/solicitor on finalisation.
- **Processors:** reliance on sub-processors' published security/DPA documentation.

Step 4 — Assess necessity and proportionality

- **Lawful basis (as Controller, own data):** contract, legal obligation, legitimate interests (see [ROPA](#)).
- **Lawful basis (clinical data, as Processor):** determined by the therapist-Controller — typically Art 9(2)(h) and/or explicit consent.
- **Data minimisation:** only data needed to run a practice is collected; health data is recorded at the therapist's discretion; reminders contain practitioner + time, not clinical content; analytics are cookieless.
- **Function creep prevention:** Faresay contractually undertakes not to use client data for its own purposes, marketing, or AI training (DPA §3.3).
- **Data-subject rights:** supported via in-product export/deletion and Processor assistance to the Controller (DPA §7).

Step 5 — Identify and assess risks

#	Risk to individuals	Likelihood	Severity	Overall
R1	Unauthorised access to health data (external attacker)	Possible	Severe	High
R2	Therapist account compromise (weak/stolen credentials)	Possible	Severe	High
R3	A therapist sees another therapist's clients (authorisation flaw)	Remote	Severe	Medium
R4	Data exposure via a sub-processor / international transfer	Remote	Severe	Medium
R5	Reminder sent to wrong recipient (email/SMS)	Possible	Moderate	Medium
R6	Excessive retention after a client leaves a practice	Possible	Moderate	Medium
R7	Re-identification via analytics	Remote	Moderate	Low

Step 6 — Measures to reduce risk

#	Measure	Effect	Residual
R1	TLS in transit; AES-256 at rest; least-privilege access; security logging; pen-test pre/post scaled launch [target]	Reduce	Low–Medium
R2	Clerk auth with strong passwords + MFA ; session controls	Reduce	Low–Medium
R3	Application-level tenant isolation; authorisation checks on every client/session route; code review	Reduce	Low
R4	UK/EEA primary storage; UK IDTA / SCCs for any transfer; sub-processor DPAs; 14-day change notice	Reduce	Low
R5	Confirmed contact fields; managed-client flows; reminders carry minimal content	Reduce	Low

#	Measure	Effect	Residual
R6	Retention policy: export window + deletion timeline (DPA §11); per-client deletion	Reduce	Low
R7	Cookieless analytics (Plausible) — no personal data collected	Eliminate	Negligible

Step 7 — Sign-off and outcomes

Item	Detail
Residual risk	Acceptable — residual risks assessed Low after the measures above
Prior consultation with ICO required?	Only if a high residual risk cannot be mitigated — not anticipated .
DPO advice	to be confirmed
Approved by	Joel Harvey, Founder — 25 June 2026
Integrated into project?	Measures tracked in the Risk Register and TOMs .
Next review	to be confirmed

Draft prepared to support a UK launch. Risk scoring and sign-off to be validated by the DPO / data-protection solicitor before reliance.

Security & Data Protection Policy

⚠️ DRAFT v0.1 — for security + legal review. NOT legal advice. Must be validated before use. Last updated: 26 June 2026

Faresay Security & Data Protection Policy

Faresay operates an online therapy / mental-health marketplace that connects clients with licensed mental-health professionals. Because the Platform handles **mental-health information** — among the most sensitive categories of personal data that exist — Faresay applies a correspondingly high standard of security and data protection across its people, processes and technology.

This policy sets out the controls Faresay maintains to protect the confidentiality, integrity and availability of the data it processes. It is written to support:

- **United Kingdom (now):** the **UK GDPR** (in particular **Article 32 — Security of processing**) and the **Data Protection Act 2018**; clinical data is **special-category** health data; Faresay is registered with the **ICO**.
- **United States (planned expansion, all 50 states + DC):** the **HIPAA Security Rule** (45 CFR Part 160 and Part 164, Subparts A and C) and the **HIPAA Breach Notification Rule**, plus state health-privacy and consumer-protection laws (e.g. Washington **My Health My Data Act**, California **CCPA/CPRA**).

This policy should be read alongside the [Privacy Policy](#), the [Terms of Service](#), the [Therapist Agreement](#), and the shared [CONTEXT.md](#) fact sheet. It is an internal operational policy; client-facing commitments live in the Privacy Policy.

⚠️ **COUNSEL / SECURITY — overarching note.** This is a first draft for review. Faresay is **bootstrapped** and early-stage; several controls below describe a **target state** rather than a control that is fully implemented today. Each section flags where this is the case. Counsel and a security specialist must validate scope, the UK/EU vs US data-residency model, the controller / processor / covered-entity / business-associate analysis, and all breach-notification obligations before this policy is relied upon.

1. Purpose & scope

1.1 Purpose

This policy defines how Faresay protects the data it processes, establishes accountability for security and data protection, and demonstrates — for UK GDPR Article 32 and, on US launch, the HIPAA Security Rule — that Faresay implements appropriate technical and organisational measures (TOMs) proportionate to the risk to individuals.

1.2 Scope

This policy applies to: - All Faresay personnel: employees, founders, contractors, and any worker with access to Faresay systems or data. - All Faresay information systems and services, including the **marketplace application**, supporting infrastructure, code repositories, administrative tooling, and corporate accounts (email, collaboration tools). - All data processed by Faresay, with **mental-health data / Protected Health Information (PHI)** treated as the highest-sensitivity class (Section 2). - Third parties and sub-processors that process Faresay data on Faresay's behalf (Section 9).

1.3 Role boundary

Faresay provides the **platform**; the **clinician** provides the clinical service and owns the clinical relationship and clinical record (see [CONTEXT.md](#) and the [Therapist Agreement](#)). The split of data-protection roles — controller / processor / joint controller (UK/EU) and covered entity / business associate (US HIPAA) — is being finalised by counsel and affects which obligations in this policy fall on Faresay versus the clinician. ⚠️ **COUNSEL**.

2. Data classification

Faresay classifies data so that controls are applied proportionately. Mental-health data is always treated at the highest level.

Class	Examples	Handling baseline
Class 1 — Highest sensitivity: mental-health data / PHI / special-category health data	The fact a person is seeking or receiving therapy; intake/assessment information; session-related clinical information and notes; safeguarding/crisis information; messages relating to care.	Strict least-privilege access; encryption in transit and at rest; full audit logging; no use in non-production environments; breach-notification regimes apply (Section 14).
Class 2 — Confidential	Account & identity data; authentication data; payment-related data; clinician licensing/verification data; internal financials; security configuration.	Role-based access; encryption in transit and at rest; logged access.
Class 3 — Internal	Internal documents, non-sensitive operational data.	Access limited to personnel; standard controls.
Class 4 — Public	Marketing pages, published clinician profile information the clinician has agreed to publish.	Integrity controls; no confidentiality requirement.

Notes: - **Mental-health data is inherently sensitive**. Even the bare fact that an individual is a Faresay client is Class 1 data and must be protected accordingly. - Where Faresay acts as a **processor / business associate** for clinical records, the clinician (as controller / covered entity) sets the lawful basis and permitted uses; Faresay processes only on documented instructions. ⚠️ **COUNSEL**. - Payment card data: Faresay intends to use a **PCI-DSS-compliant payment processor** so that Faresay does not store raw card data. [PLACEHOLDER: payment processor]. ⚠️ **SECURITY** — confirm cardholder-data flows and PCI scope.

3. Governance, roles & responsibilities

3.1 Accountability

Faresay's leadership (founder/management) is ultimately accountable for information security and data protection and for approving this policy.

3.2 Key roles

- **Security Lead** — [PLACEHOLDER: named individual]. Owns this policy, the risk treatment plan, vendor security review, incident response coordination, and the security roadmap. In an early-stage bootstrapped company this

may initially be a founder; ⚠️ **SECURITY** — confirm whether a fractional/virtual CISO or external security advisor is engaged.

- **Data Protection Officer (DPO) / privacy lead** — Joel Harvey, Founder — support@faresay.com. ⚠️ **COUNSEL** — confirm whether a statutory **DPO is required** under UK GDPR Art 37 (likely triggered by large-scale processing of special-category health data) and, if so, appoint and register the DPO with the ICO. The DPO/privacy lead owns DPIAs, records of processing (Art 30), data-subject rights handling, and the ICO relationship.
- **HIPAA Security Official & Privacy Official (US)** — ⚠️ **COUNSEL** — the HIPAA Security Rule requires a designated **Security Official** (45 CFR §164.308(a)(2)) and the Privacy Rule a **Privacy Official**; these must be named before US launch if Faresay is a covered entity or business associate.
- **All personnel** — responsible for following this policy, completing training, reporting incidents promptly, and protecting credentials and devices.

3.3 Decision-making & review

Security risks are tracked in the [risk-register.md](#). Material security decisions, exceptions and accepted risks are recorded with an owner and review date.

4. Access control

4.1 Principles

- **Least privilege** — personnel receive the minimum access required for their role; broad/admin access is the exception, justified and time-bound where possible.
- **Role-Based Access Control (RBAC)** — access is granted by role, not individually, wherever the platform and tooling support it.
- **Need-to-know for Class 1 data** — access to mental-health data / PHI is restricted to roles with a genuine operational need and is logged (Section 11).
- **Separation of duties** — production access, code-deployment rights and security administration are separated where headcount allows; ⚠️ **SECURITY** — document compensating controls given small team size.

4.2 Authentication

- **Multi-factor authentication (MFA)** is required for all administrative and production-system access, code repositories, the hosting provider, DNS, email, and any system holding Class 1 or Class 2 data.
- Strong, unique passwords managed via a password manager; no shared accounts for administrative access.
- ⚠️ **SECURITY** — adopt SSO with enforced MFA across corporate SaaS where feasible. [PLACEHOLDER: identity provider / SSO].

4.3 Joiner / mover / leaver (JML)

- **Joiner** — access provisioned by role on a documented request; principle of least privilege; security onboarding and confidentiality agreement before access to Class 1/Class 2 data.
- **Mover** — access reviewed and adjusted on role change; entitlements no longer needed are revoked.
- **Leaver** — all access revoked promptly on departure (target: same business day; immediately for involuntary departures); credentials rotated; devices returned/wiped.

- **Periodic access reviews** — entitlements (especially to Class 1 data and admin roles) reviewed at least [PLACEHOLDER: quarterly] and stale access removed.
-

5. Encryption & key management

5.1 In transit

- **TLS** (current secure version, [PLACEHOLDER: TLS 1.2+ / 1.3]) enforced for all connections to the Platform and between services. HTTP redirected to HTTPS; HSTS enabled.
- TLS termination and edge security are provided via **Cloudflare** (DNS) and **Vercel** (web hosting) for the marketplace web app. ⚠️ **SECURITY** — confirm certificate management and that all backend/API and database connections are also encrypted.

5.2 At rest

- Class 1 and Class 2 data encrypted at rest using strong, industry-standard algorithms (e.g. AES-256). [PLACEHOLDER: database / storage provider and their at-rest encryption].
- Backups encrypted at rest (Section 13).

5.3 Key management

- Encryption keys and secrets (API keys, database credentials) managed via a dedicated secrets manager / platform environment-variable store, never committed to source control. [PLACEHOLDER: secrets management solution].
 - Key access restricted, logged, and keys rotated on a defined schedule and on suspected compromise. ⚠️ **SECURITY** — define key-rotation cadence and ownership.
-

6. Infrastructure, hosting & data residency

6.1 Known stack (per CONTEXT.md)

- **Marketplace application** — the core product.
- **Web hosting: Vercel.**
- **DNS: Cloudflare.**
- **Application database / backend services / file storage:** [PLACEHOLDER: provider(s) — not specified in CONTEXT]. ⚠️ **SECURITY** — document the full inventory: database, object storage, email, payments, video/session delivery, analytics, error monitoring.

6.2 Data residency ⚠️ COUNSEL

- **UK/EU (now):** confirm that personal data — especially Class 1 mental-health data — is stored and processed in the **UK/EU**, or that a lawful **international transfer** mechanism is in place (UK IDTA / Addendum, EU SCCs, adequacy). Vercel and Cloudflare operate globally; **region configuration and routing must be verified**, since default configurations may route or store data outside the UK/EU. ⚠️ **COUNSEL.**
- **US (planned):** determine US data-residency requirements and whether US client data must remain in the US. Confirm that any provider handling PHI will sign a **HIPAA Business Associate Agreement** and offers a HIPAA-

eligible configuration (Section 9). ⚠️ **COUNSEL**.

- ⚠️ **SECURITY** — maintain a data-flow map showing where each data class is stored, processed and transmitted, including sub-processor locations.

7. Secure software development lifecycle (SSDLC)

- **Secure by design** — security and privacy considered from design; a **DPIA** is completed for high-risk processing of special-category data (UK GDPR Art 35). ⚠️ **COUNSEL**.
- **Source control & review** — code held in version control ([PLACEHOLDER: GitHub/GitLab]); changes via pull request with **peer review** before merge to production where headcount allows; protected main branch.
- **Secrets hygiene** — secret scanning enabled; no credentials in code or logs.
- **Dependency & supply-chain security** — automated dependency scanning (e.g. Dependabot/equivalent) and timely patching of vulnerable libraries.
- **Application security** — input validation, output encoding, parameterised queries, authentication/authorisation checks, protection against OWASP Top 10 risks.
- **Environment separation** — production, staging and development environments separated; **production Class 1 data is never used in non-production environments** (use synthetic/anonymised test data).
- **CI/CD** — automated build, test and deploy pipelines with access controls on deployment. [PLACEHOLDER: CI/CD tooling].
- ⚠️ **SECURITY** — formalise security testing (SAST/DAST) in the pipeline as the team matures.

8. (Reserved — see Section 9 for third-party management)

Section intentionally consolidated into Section 9.

9. Third-party & sub-processor management

9.1 Inventory & due diligence

- Faresay maintains a register of all sub-processors / vendors that process Faresay data, the data classes involved, and their locations. [PLACEHOLDER: sub-processor register].
- Vendors handling Class 1/Class 2 data are subject to security due diligence proportionate to risk (e.g. SOC 2 / ISO 27001 evidence, security questionnaire).

9.2 Contracts — DPAs and BAAs ⚠️ **COUNSEL**

- **UK/EU: a Data Processing Agreement (DPA)** compliant with UK GDPR Art 28 is required with every processor handling personal data, including appropriate international-transfer terms (IDTA/SCCs).
- **US / HIPAA:** where Faresay is a **covered entity or business associate** and a vendor will create, receive, maintain or transmit **PHI**, a **HIPAA Business Associate Agreement (BAA)** is **mandatory** before any PHI is shared. This includes hosting, storage, email, video/session, and analytics providers that touch PHI. ⚠️ **COUNSEL**.

- **⚠ SECURITY / COUNSEL** — confirm which of the current providers (Vercel, Cloudflare, and the to-be-confirmed database/email/payment/video providers) will sign a BAA and offer a HIPAA-eligible plan; some default plans do **not** include a BAA. Do not transmit PHI to any provider without a signed BAA.
-

10. (Reserved)

See Section 11.

11. Logging, monitoring & audit trails

- **Audit logging** — access to and changes affecting Class 1 (mental-health/PHI) and Class 2 data are logged with user identity, timestamp and action, to support HIPAA audit-control requirements (45 CFR §164.312(b)) and UK GDPR accountability. **⚠ SECURITY** — confirm application-level audit logging covers reads of clinical data, not just writes.
 - **Security monitoring** — infrastructure, authentication and application logs collected and reviewed; alerting on suspicious activity (e.g. failed-login spikes, privilege changes). [PLACEHOLDER: logging/monitoring/SIEM tooling].
 - **Log protection & retention** — logs protected against tampering and unauthorised access, and retained for [PLACEHOLDER: retention period — align with HIPAA's 6-year documentation retention and ICO guidance]. **⚠ COUNSEL.**
 - **No sensitive data in logs** — Class 1 content and secrets must not be written to application logs.
-

12. Vulnerability management & penetration testing

- **Patch management** — operating systems, dependencies and platform components kept up to date; critical vulnerabilities remediated on a defined SLA (target: [PLACEHOLDER: e.g. critical within 7 days]).
 - **Vulnerability scanning** — regular automated scanning of dependencies and, where applicable, infrastructure.
 - **Penetration testing** — independent penetration test of the Platform before US launch and at least annually / on major change thereafter. [PLACEHOLDER: pentest provider/cadence]. **⚠ SECURITY.**
 - **Coordinated disclosure** — a channel for reporting vulnerabilities ([PLACEHOLDER: security@ address]) and a triage process.
-

13. Backup, disaster recovery & business continuity

- **Backups** — regular automated backups of production data, encrypted at rest, with backup integrity/restore testing. [PLACEHOLDER: backup solution, frequency, retention].
 - **Recovery objectives** — defined **RPO** and **RTO**: [PLACEHOLDER: RPO/RTO targets]. **⚠ SECURITY.**
 - **Disaster recovery** — documented procedure to restore service after major failure; reliance on managed providers (Vercel, Cloudflare, and the database provider) is noted, with their resilience/SLA considered.
 - **Business continuity** — plan for continuity of essential operations (including client safety and crisis pathways — see [crisis-safeguarding-policy.md]) during disruption. **⚠ CLINICAL / SECURITY.**
-

14. Incident response & breach notification ⚠️ COUNSEL

14.1 Incident response

- A documented incident-response process covers detection, triage, containment, eradication, recovery, notification and post-incident review.
- All personnel must report suspected incidents immediately to the Security Lead. [PLACEHOLDER: reporting channel].
- Incidents are logged and assessed for severity and for whether a notifiable personal-data breach has occurred.

14.2 UK breach notification

- Where a personal-data breach is likely to result in a risk to individuals' rights and freedoms, Faresay (or the responsible controller) notifies the **ICO without undue delay and within 72 hours** of becoming aware (UK GDPR Arts 33–34).
- Where the breach is likely to result in a **high risk**, affected individuals are notified without undue delay.
- ⚠️ **COUNSEL** — confirm Faresay's notification role given the controller/processor split; a processor must notify the controller without undue delay rather than the regulator directly.

14.3 US breach notification (on US launch)

- The **HIPAA Breach Notification Rule** (45 CFR §§164.400–414) requires, for breaches of unsecured PHI, notification to affected individuals (generally within **60 days**), to **HHS**, and — for larger breaches — to the media; business associates must notify the covered entity.
 - **State Attorney General / state breach-notification laws** impose additional and varying obligations across the 50 states + DC (timing, content, AG notice), and consumer-health-data laws (e.g. Washington **My Health My Data Act**) may add further requirements.
 - ⚠️ **COUNSEL** — build a state-by-state breach-notification matrix before US launch; obligations and deadlines vary materially by state.
-

15. Personnel security

- **Background checks** — pre-engagement screening proportionate to role and access to Class 1 data, subject to local law (e.g. UK DBS where appropriate; US background checks). ⚠️ **COUNSEL** — confirm lawful basis/limits for screening in each jurisdiction. (Clinician licensing/verification is handled separately under the [Therapist Agreement](#) and clinical governance.)
 - **Confidentiality agreements** — all personnel with access to Class 1/Class 2 data sign confidentiality / NDA terms.
 - **Security & data-protection training** — at onboarding and at least annually, including phishing awareness and handling of mental-health data; HIPAA workforce training before US launch (45 CFR §164.308(a)(5)).
 - **Sanctions** — defined consequences for policy violations.
-

16. Physical security

- Faresay is a [PLACEHOLDER: remote-first?] organisation relying primarily on **cloud-hosted** infrastructure; physical data-centre security is inherited from providers (Vercel, Cloudflare, and the database provider) and evidenced via their certifications (e.g. SOC 2 / ISO 27001).
 - **Endpoint security** — personnel devices accessing Faresay data must use full-disk encryption, screen lock, current OS/security updates, and reputable endpoint protection; lost/stolen devices reported immediately.
 - **Workspace** — Class 1 data must not be viewed in insecure public settings; clear-screen/clear-desk discipline applies.
 - **⚠ SECURITY** — confirm any office/physical access controls and mobile-device management (MDM). [PLACEHOLDER: MDM solution].
-

17. Acceptable use

- Faresay systems and data are used only for authorised business purposes.
 - No sharing of credentials; no exporting or copying Class 1 data outside approved systems; no use of unapproved tools/services (including AI tools) to process Class 1/Class 2 data without authorisation.
 - Personnel must protect devices and credentials, follow this policy, and report security concerns.
 - **⚠ SECURITY** — define an approved-tools list and rules for any AI/LLM tooling that could expose mental-health data.
-

18. Mapping to HIPAA Security Rule safeguards **⚠ COUNSEL**

The following maps this policy to the HIPAA Security Rule safeguard categories. This is a **planning aid for US launch**, not a confirmation of compliance, and assumes Faresay is a covered entity and/or business associate (to be confirmed). **⚠ COUNSEL**.

18.1 Administrative safeguards (45 CFR §164.308)

- Security Management Process / risk analysis & risk management — Sections 3, 12; [risk-register.md].
- Assigned Security Responsibility (Security Official) — Section 3. **⚠ COUNSEL** (appoint).
- Workforce Security & Information Access Management — Sections 4, 15.
- Security Awareness & Training — Section 15.
- Security Incident Procedures — Section 14.
- Contingency Plan (backup, DR, emergency mode) — Section 13.
- Evaluation — Section 21 (periodic review).
- Business Associate Contracts — Section 9 (BAAs).

18.2 Physical safeguards (45 CFR §164.310)

- Facility Access Controls — Section 16 (inherited from providers).
- Workstation Use & Security — Sections 16, 17.
- Device & Media Controls (disposal, re-use, accountability) — Sections 16, 19.

18.3 Technical safeguards (45 CFR §164.312)

- Access Control (unique user ID, automatic logoff, encryption/decryption) — Sections 4, 5.
- Audit Controls — Section 11.
- Integrity — Sections 5, 7, 11.
- Person/Entity Authentication — Section 4 (MFA).
- Transmission Security — Section 5 (TLS).

⚠️ **COUNSEL / SECURITY** — a full HIPAA Security Rule gap assessment, risk analysis and documentation set must be completed before US launch.

19. Data retention, minimisation & secure disposal

- **Minimisation** — Faresay collects and retains only the data necessary for the purposes set out in the [Privacy Policy](#).
- **Retention** — data retained per a defined retention schedule. Note that **clinical records are typically owned and retained by the clinician** under professional/clinical-records rules; Faresay's retention of platform data is separate. [PLACEHOLDER: retention schedule]. ⚠️ **COUNSEL / CLINICAL**.
- **Secure disposal** — data securely deleted/anonymised at end of retention; media sanitised or providers' certified-destruction processes relied upon; backups age out per the backup retention policy.
- ⚠️ **COUNSEL** — reconcile retention with UK GDPR storage limitation, HIPAA's 6-year documentation retention, and clinical-records retention rules, which differ.

20. Alignment to a recognised framework (future goal)

Faresay's security programme is structured to align over time with a recognised framework. As a **bootstrapped, early-stage** company, formal certification is a **future goal** rather than a current state. Candidate frameworks: - **SOC 2 (Type II)** — likely the most market-relevant signal for a US healthtech marketplace and enterprise/clinician trust. - **ISO/IEC 27001** — internationally recognised ISMS certification. - **NIST Cybersecurity Framework / NIST 800-53** — useful as a control reference.

⚠️ **SECURITY** — agree the target framework and a realistic roadmap (likely: implement controls now, pursue SOC 2 around US launch).

21. Policy review cadence

- This policy is reviewed at least **annually** and after any major change to systems, processing, regulation, or following a significant incident.
- The Security Lead owns the review; the DPO/privacy lead and counsel review data-protection and regulatory aspects.
- Version history and review dates are maintained below.

Version	Date	Author	Notes
v0.1 (DRAFT)	26 June 2026	[PLACEHOLDER: author]	Initial draft for security + legal review.

End of draft. ⚠️ This document is a v0.1 draft and must be validated by qualified security and legal professionals, with all [PLACEHOLDER: ...] items resolved, before it is relied upon or published.

Clinical Governance Policy

⚠️ DRAFT v0.1 — for clinical advisor + legal review. NOT clinical or legal advice. Must be validated by a qualified clinical lead before use. Last updated: 26 June 2026

Faresay Clinical Governance Policy

1. Purpose and Scope

1.1 Purpose

1.1.1 This Clinical Governance Policy (the "**Policy**") sets out the framework through which Faresay seeks to assure the quality and safety of the clinical services delivered by independent mental-health professionals through the Faresay marketplace platform.

1.1.2 The Policy describes how clinical standards are set, how practitioners are verified and monitored, how clinical quality and safety are overseen, and how clinical risks, incidents, and complaints are managed. Its objective is to support safe, ethical, evidence-informed care while preserving each practitioner's independent professional clinical judgement.

1.1.3 Faresay provides the **technology and marketing platform** (software, discovery, scheduling, payments, client acquisition, and administrative support). Faresay does **not** provide clinical services, does not employ practitioners to provide clinical services, and is not a party to the clinical relationship. The practitioner provides the clinical service and owns the clinical relationship and the clinical record. This Policy is a governance and quality-assurance framework for the marketplace; it is not, and must not be read as, Faresay directing, controlling, or interfering with any practitioner's clinical judgement.

⚠️ COUNSEL: The boundary between (a) legitimate marketplace quality/safety governance and (b) the practice of psychology/medicine or the exercise of clinical control matters for corporate-practice-of-medicine / corporate-practice-of-psychology (CPOM) doctrine, fee-splitting rules, and the MSO / "friendly PC" structure. The whole Policy must be reviewed by US healthcare-regulatory counsel (per state) and UK counsel to confirm it does not, in substance or appearance, transfer clinical control to a non-clinical entity. See `therapist-agreement.md` clauses 2.4–2.6.

1.2 Scope

1.2.1 This Policy applies to:

- all mental-health practitioners onboarded to and delivering services through the Platform ("Practitioners");
- Faresay personnel and contractors involved in clinical governance, onboarding, verification, trust and safety, and support;
- the clinical advisory function described in Section 3;
- where applicable in a US state, any professional entity ("friendly PC" / professional corporation) through which clinical services are contracted or supervised.

1.2.2 **Territory.** This Policy applies to Faresay's current operations in the **United Kingdom** and to its planned expansion into the **United States (the 50 states and the District of Columbia)**. Where a requirement differs

between the UK and the US, or between US states, this is flagged. US-specific requirements take effect only on launch in the relevant state and are subject to validation by counsel and the clinical lead.

1.2.3 This Policy sits alongside and should be read with: the Therapist Agreement ([therapist-agreement.md](#)), the Crisis / Safeguarding Policy ([crisis-safeguarding-policy.md](#)), the Privacy Policy ([privacy-policy.md](#)), the Security & Data Protection Policy ([security-data-protection-policy.md](#)), and the Terms of Service ([terms-of-service.md](#)).

1.3 Status and ownership of this Policy

1.3.1 This Policy is owned by the Clinical Lead (see Section 3) and approved by [PLACEHOLDER: approving body — e.g. Faresay board / clinical advisory committee].

1.3.2 ⚠️ CLINICAL: This draft has been prepared without a qualified clinical lead in post. Every clinical standard, threshold, and exclusion in this Policy must be reviewed, corrected, and validated by a qualified clinical lead (and, where US-specific, by US-licensed clinical advisors) before the Policy is relied upon.

2. Definitions

In this Policy:

- **"Client"** means an individual who books, accesses, or receives clinical services from a Practitioner via the Platform.
- **"Clinical Lead"** means the qualified, suitably-registered senior clinician accountable for clinical governance at Faresay, as described in Section 3.
- **"Clinical Advisory Function"** means the clinical advisory committee, panel, or named advisors supporting the Clinical Lead, as described in Section 3.
- **"Clinical Record"** means the clinical notes, assessments, treatment plans, correspondence, and other professional records a Practitioner creates in connection with the care of a Client.
- **"Clinical Services"** means the regulated mental-health, psychotherapy, or counselling services a Practitioner provides to Clients.
- **"Compact"** means an interstate licensure compact relevant to mental-health practice, including PSYPACT (psychology), the Counseling Compact, and the Social Work Licensure Compact.
- **"DBS"** means the UK Disclosure and Barring Service.
- **"Friendly PC" / "PC"** means a US professional corporation or professional entity through which clinical services may be contracted or supervised in a corporate-practice state.
- **"MSO"** means a management services organisation (Faresay's commercial / non-clinical entity in a US corporate-practice structure).
- **"Practitioner"** means an independent, licensed mental-health professional onboarded to deliver Clinical Services through the Platform.
- **"Professional Body"** means the relevant statutory regulator or professional membership/accreditation body for a Practitioner (see Section 18).
- **"Routine Outcome Measure" / "ROM"** means a validated, standardised measure used to track client symptoms, functioning, or progress over the course of care.
- **"Scope of Practice"** means the range of clinical activities a Practitioner is qualified, trained, competent, and licensed/registered to perform.
- **"State"**, in the US context, means a US state or the District of Columbia.

- **"Telehealth"** means the delivery of Clinical Services remotely by video, voice, or messaging.
-

3. Governance Structure and Accountability

3.1 Principle of clinical leadership

3.1.1 ⚠️ CLINICAL: Clinical governance at Faresay must be led by a qualified clinician, not by commercial or product staff. Faresay will appoint a **Clinical Lead** who is professionally responsible and accountable for the matters in this Policy.

3.1.2 The Clinical Lead must hold a current, valid, unrestricted licence/registration with a relevant Professional Body and have appropriate seniority and experience in mental-health practice and clinical governance.

[PLACEHOLDER: named Clinical Lead, qualifications, registration number.]

⚠️ COUNSEL / ⚠️ CLINICAL: In US corporate-practice states, the senior clinical authority may need to sit within (or be appointed by) the professional entity ("friendly PC"), not the MSO, to avoid the appearance of a non-clinical entity controlling clinical decisions. The reporting line and appointment mechanism for the Clinical Lead in the US structure must be designed with counsel.

3.2 Role of the Clinical Lead

3.2.1 The Clinical Lead is responsible for: owning and maintaining this Policy and related clinical standards; overseeing Practitioner onboarding, verification, and credentialing criteria (Section 4); defining scope-of-practice and exclusion criteria (Sections 6 and 15); overseeing quality assurance and outcome monitoring (Section 11); chairing or convening the Clinical Advisory Function; overseeing clinical incident review (Section 12) and the clinical complaints route (Section 13); and advising the business on clinical risk.

3.2.2 The Clinical Lead may delegate operational tasks but remains accountable for the clinical governance framework.

3.3 Clinical Advisory Function

3.3.1 ⚠️ CLINICAL: Faresay will establish a **Clinical Advisory Function** — a committee, panel, or set of named advisors — to support the Clinical Lead. Its composition, terms of reference, quorum, and meeting cadence are to be defined. [PLACEHOLDER: committee structure and members.]

3.3.2 The Clinical Advisory Function should, at minimum, include clinicians representing the main professional disciplines on the Platform (e.g. clinical/counselling psychology, psychotherapy, counselling, clinical social work) and, on US expansion, US-licensed clinicians familiar with the relevant state and professional standards. ⚠️ CLINICAL: confirm the appropriate disciplinary mix and whether independent (external) advisors are required.

3.3.3 Indicative responsibilities: reviewing and endorsing clinical standards and exclusion criteria; periodic case review and audit (Section 11); reviewing serious clinical incidents and trends (Section 12); and advising on emerging clinical risks and evidence.

3.4 Non-clinical roles

3.4.1 Faresay's trust-and-safety, onboarding/verification, support, data-protection, and engineering teams support clinical governance operationally (for example, by running verification checks and maintaining systems) but do not make clinical decisions. Clinical decisions about individual clients rest solely with the treating Practitioner.

3.4.2 A roles-and-responsibilities matrix is set out in Section 17.

4. Practitioner Onboarding and Verification

4.1 Principle

4.1.1 No Practitioner may deliver Clinical Services through the Platform until they have completed onboarding and verification and have been approved. Onboarding establishes that a Practitioner is appropriately qualified, licensed/registered, insured, and fit to practise.

4.2 Licensure / registration verification

4.2.1 **UK.** Faresay will verify that the Practitioner holds a current, valid, unrestricted registration or accredited membership with the relevant UK Professional Body (e.g. HCPC for practitioner psychologists; BACP, UKCP, or BABCP for counsellors/psychotherapists, as applicable). ⚠️ **CLINICAL:** confirm the minimum acceptable registers/accreditations Faresay will accept, and whether unaccredited counsellors are excluded.

4.2.2 **US — licence in the client's state.** A Practitioner must hold a current, valid, unrestricted licence to practise in the **State in which the Client is physically located at the time of the session**, consistent with the licensing requirements of that State's board. Faresay must verify licensure against the relevant State licensing board for each State in which the Practitioner is offered to clients.

⚠️ **COUNSEL / ⚠️ CLINICAL:** Telehealth licensure generally turns on where the *client* is located, not the practitioner. Per-State rules, exceptions, and any temporary/telehealth registrations must be confirmed by counsel and reflected in the platform's matching logic so a Practitioner can only be booked by clients in States where they are authorised.

4.2.3 **Interstate compacts.** Where a Practitioner holds authority under an applicable Compact, Faresay may verify and rely on that authority for the relevant States, subject to the compact's terms:

- **PSYPACT** for qualifying psychologists;
- the **Counseling Compact** for qualifying professional counsellors;
- the **Social Work Licensure Compact** for qualifying clinical social workers.

⚠️ **COUNSEL:** Compact participation, the scope of authority each compact grants (e.g. telehealth vs temporary in-person practice), member-State status, and the home-State requirement vary and change over time. Reliance on a compact for any given Client/State must be validated by counsel and re-checked as compacts and State participation evolve.

4.2.4 Verification must capture and store, at minimum: the licensing/registering body, licence/registration number, jurisdiction(s), licence type and scope, issue and expiry dates, and current standing. [PLACEHOLDER: verification provider / method — e.g. primary-source verification vendor.]

4.3 References

4.3.1 Faresay will obtain and review [PLACEHOLDER: number] professional reference(s) appropriate to the role. ⚠️ **CLINICAL:** confirm reference requirements (number, source, whether clinical-supervisor references are required).

4.4 Background / DBS checks

4.4.1 **UK.** Faresay will require an appropriate criminal-records / suitability check. ⚠️ **COUNSEL / ⚠️ CLINICAL:** confirm the correct DBS level for remote, non-regulated-activity mental-health work (basic vs standard vs enhanced), and whether the work meets the definition of "regulated activity" — this affects DBS eligibility and barred-list checks. Do not assert a DBS level until confirmed.

4.4.2 **US.** Faresay will require an appropriate background check consistent with applicable State and federal law. ⚠️ **COUNSEL:** background-check scope, consent (FCRA), and use of results vary by State and are subject to fair-chance / "ban-the-box" and adverse-action rules; confirm with counsel before defining the standard.

4.5 Professional indemnity insurance

4.5.1 Each Practitioner must hold and maintain professional indemnity / professional liability (malpractice) insurance appropriate to their discipline, jurisdiction(s) of practice, and the telehealth services delivered, at coverage limits no lower than [PLACEHOLDER: minimum limits]. Faresay will verify cover at onboarding and on renewal.

⚠️ **COUNSEL** / ⚠️ **CLINICAL:** Minimum coverage limits, whether telehealth and the relevant US States are within the policy's scope, and any requirement for Faresay to be named/additional insured must be set with counsel and the clinical lead. UK and US norms differ.

4.6 Fitness and declarations

4.6.1 At onboarding and on renewal, Practitioners must declare any past or pending fitness-to-practise proceedings, licence conditions/restrictions/suspensions, criminal matters, and material complaints or claims. Material non-disclosure may result in removal from the Platform.

4.7 Ongoing licence / sanction monitoring

4.7.1 Verification is **not** a one-time event. Faresay will operate ongoing monitoring of each Practitioner's continued eligibility, including:

- monitoring **licence/registration expiry** and requiring evidence of renewal before lapse;
- periodic re-verification of standing with each relevant Professional Body / State board on a defined cadence ([PLACEHOLDER: e.g. at least annually, and on renewal]);
- monitoring for **disciplinary actions, sanctions, conditions, suspensions, or restrictions**, including (US) checks against State board actions and relevant national databases. ⚠️ **COUNSEL:** confirm permissible/required sources (e.g. NPDB access eligibility, OIG/SAM exclusion lists, State board disciplinary feeds) — access to some databases is restricted to certain entity types.
- monitoring lapses or changes to professional indemnity insurance.

4.7.2 On discovery of an expiry, sanction, restriction, or loss of insurance affecting a Practitioner's eligibility for any State/jurisdiction, Faresay will suspend the Practitioner's availability for the affected jurisdiction(s) pending review by the Clinical Lead, and manage any client-continuity-of-care implications under Sections 6, 12, and the Crisis / Safeguarding Policy ([crisis-safeguarding-policy.md](#)).

5. Standards of Care and Evidence-Based Practice

5.1 Practitioners are expected to deliver care that is consistent with current, recognised, evidence-based or evidence-informed practice for the presenting need and client population, within the limits of telehealth delivery.

5.2 Practitioners must comply with the ethical and practice standards of their Professional Body / licensing board (Section 18), including standards specific to remote/telehealth delivery.

5.3 ⚠️ **CLINICAL:** Confirm whether Faresay will reference or require adherence to specific clinical guidelines (e.g. NICE guidance in the UK; recognised US professional-body practice guidelines) or remain at the level of

"consistent with the Practitioner's professional standards and competence." Avoid setting standards that could be read as Faresay directing clinical care.

5.4 Practitioners must keep their knowledge and skills current (see Section 10) and practise within their competence (Section 6).

6. Scope of Practice and Competence

6.1 Practising within competence

6.1.1 Practitioners must only provide Clinical Services that fall within their **Scope of Practice** — that is, services they are qualified, trained, competent, licensed/registered, and (where required) supervised to provide — and only with client populations and presentations they are competent to treat.

6.1.2 Practitioners must not work beyond their competence and must refer or signpost a Client elsewhere where the Client's needs fall outside the Practitioner's competence or outside what can be safely delivered on this Platform.

6.2 Presentations unsuitable for the Platform

6.2.1 ⚠️ CLINICAL: Faresay operates a **cash-pay telehealth marketplace**. Certain presentations and acuity levels are not appropriate for this setting and must be excluded from, or escalated out of, Platform-based care. These include (indicatively, to be finalised by the Clinical Lead):

- active suicidal or homicidal crisis, or imminent risk of serious harm to self or others;
- presentations requiring urgent, emergency, or in-person crisis intervention or hospitalisation;
- conditions requiring a level or intensity of care that telehealth cannot safely provide (e.g. acute psychosis requiring stabilisation, severe/high-risk eating disorders requiring medical monitoring, acute withdrawal/detoxification);
- circumstances where the Client cannot be safely treated remotely (e.g. no safe, private environment; jurisdictional/age limits).

6.2.2 The detailed clinical thresholds, screening, and the response when an excluded presentation is identified before or during care (including safe referral, escalation, and signposting to emergency services) are governed by the **Crisis / Safeguarding Policy ([crisis-safeguarding-policy.md](#))**, which this Policy cross-references and must be read with. ⚠️ CLINICAL: ensure the exclusion list here and the screening/escalation pathways in the Crisis / Safeguarding Policy are mutually consistent.

7. Informed Consent

7.1 Before Clinical Services begin, the Practitioner is responsible for obtaining the Client's informed consent to treatment, consistent with the Practitioner's professional and legal obligations.

7.2 Informed consent should cover, at minimum: the nature, format, and limits of telehealth delivery; the likely approach and any material risks/benefits and alternatives; confidentiality and its limits (including duty-to-warn / safeguarding disclosures — see [crisis-safeguarding-policy.md](#)); how clinical records are kept and by whom; fees and the cash-pay model; and arrangements for crisis/out-of-hours situations and what to do in an emergency.

7.3 ⚠️ CLINICAL / ⚠️ COUNSEL: Informed-consent content and documentation requirements differ by jurisdiction and discipline (e.g. specific US State telehealth-consent statutes; capacity and consent for minors; emergency-

contact and "patient location" attestations). Confirm jurisdiction-specific consent requirements and whether consent must be documented in a prescribed form.

7.4 Consent to **clinical** treatment (the Practitioner's responsibility) is distinct from consent to **data processing** by Faresay as a platform, which is addressed in the Privacy Policy (`privacy-policy.md`).

8. Clinical Record-Keeping Standards

8.1 The Practitioner / PC holds the clinical record

8.1.1 The **clinical record is owned and held by the Practitioner** (or, in a US corporate-practice structure, by the professional entity / "friendly PC" through which the Practitioner provides care) — **not by Faresay**. The Practitioner is the controller/custodian of the Clinical Record and is responsible for creating, maintaining, retaining, securing, and (where required) disposing of it in accordance with law and professional standards. See `therapist-agreement.md` clause 2.3.

8.1.2 Faresay processes Client and platform data as a technology provider as described in the Privacy Policy (`privacy-policy.md`) and Security & Data Protection Policy (`security-data-protection-policy.md`); this is distinct from holding the Clinical Record.

⚠ COUNSEL: The controller/processor (UK GDPR) and covered-entity/business-associate (US HIPAA) characterisation of Faresay versus the Practitioner/PC for the Clinical Record and associated data must be confirmed with counsel, and may require a data processing agreement / business associate agreement. See `security-data-protection-policy.md` .

8.2 Minimum record-keeping standards

8.2.1 Practitioners must keep clinical records that are, at minimum:

- **accurate, contemporaneous, and legible** — created at or close to the time of the contact;
- **adequate** — sufficient to support continuity and the clinical decisions made, including presenting issues, assessment, risk assessment, formulation/treatment plan, consent, session notes, and any referrals or safeguarding actions;
- **secure and confidential** — stored to the security standard required by law and `security-data-protection-policy.md` ;
- **retained** for the period required by the Practitioner's jurisdiction(s) and Professional Body, and then securely disposed of.

8.2.2 [PLACEHOLDER: minimum retention periods] ⚠ CLINICAL / ⚠ COUNSEL: retention periods differ by jurisdiction, discipline, and client age (e.g. records for minors). Confirm per UK and per US State, and whether Faresay's platform tooling stores any clinical content (and if so, under what agreement).

9. Clinical Supervision and Continuing Professional Development

9.1 Practitioners must maintain clinical supervision and continuing professional development (CPD) consistent with the requirements of their Professional Body / licensing board, and must be able to evidence this on request.

9.2 ⚠ CLINICAL: Some UK modalities/bodies require ongoing supervision (e.g. BACP's supervision requirement for counsellors); US requirements vary by State and licence type (including supervision requirements for

pre-/associate-licensed clinicians). Confirm the minimum supervision and CPD standards Faresay will require, and whether pre-/associate-licensed practitioners are admitted at all and, if so, how supervision is evidenced.

9.3 Faresay does not act as the Practitioner's clinical supervisor and does not provide clinical supervision; responsibility for arranging adequate supervision rests with the Practitioner.

10. Quality Assurance and Outcome Monitoring

10.1 ⚠️ CLINICAL: Faresay will operate a clinical quality-assurance programme, overseen by the Clinical Lead and Clinical Advisory Function. The detailed design — including which measures, thresholds, and review cadences are appropriate and clinically valid — must be set by the clinical lead. Indicative components:

- **Routine Outcome Measures (ROMs):** use of validated, standardised measures (e.g. [PLACEHOLDER: instruments — to be selected by clinical lead]) to track client progress, used as a clinical tool by Practitioners rather than as a performance metric imposed by Faresay.
- **Client feedback:** structured post-session/episode feedback and satisfaction measures, and a clear route for clients to raise clinical concerns (Section 13).
- **Periodic case review / clinical audit:** sampling and review of de-identified cases or anonymised practice data against agreed standards, by the Clinical Advisory Function.
- **Practitioner quality signals:** monitoring of indicators such as cancellation/no-attend patterns, complaints, and incidents, used supportively and proportionately.

10.2 ⚠️ COUNSEL / ⚠️ CLINICAL: Outcome and case-review activity must be designed so it does not (a) involve Faresay in the practice of psychology/medicine or override clinical judgement, or (b) breach client confidentiality / data-protection law. Confirm the lawful basis, de-identification approach, and consent position for any clinical-data use with counsel; confirm clinical validity with the clinical lead.

10.3 Quality-assurance findings feed into Practitioner support, this Policy's periodic review (Section 16), and the risk register (`risk-register.md`).

11. Incident Reporting and Management

11.1 A **clinical incident** is any event in connection with Clinical Services that caused, or had the potential to cause, harm to a Client or another person — including safeguarding concerns, serious adverse events, suspected boundary or ethical breaches, data incidents affecting clinical information, and "near misses."

11.2 Practitioners and Faresay personnel must report clinical incidents promptly through [PLACEHOLDER: reporting channel / form]. Faresay will maintain an incident log and triage incidents to the Clinical Lead.

11.3 The Clinical Lead (with the Clinical Advisory Function for serious incidents) will ensure each incident is reviewed, immediate client safety is addressed, root causes and learning are identified, and corrective actions are tracked to completion. Trends are reported into quality assurance (Section 10) and the risk register (`risk-register.md`).

11.4 ⚠️ CLINICAL / ⚠️ COUNSEL: Certain incidents trigger **external** reporting or notification duties — for example to a regulator/licensing board, a safeguarding authority, an insurer, or (for data breaches) the ICO/affected individuals (UK) or under HIPAA breach-notification rules (US). The mandatory external reporting matrix must be confirmed per jurisdiction with counsel and the clinical lead. Crisis events (e.g. risk to life) are governed by the Crisis / Safeguarding Policy (`crisis-safeguarding-policy.md`).

11.5 Faresay will not retaliate against good-faith reporting of incidents or concerns.

12. Complaints Handling — Clinical Route

12.1 Faresay operates a clear route for Clients (and others) to raise concerns or complaints. Complaints are triaged into:

- **Clinical complaints** — concerning the quality, safety, or conduct of clinical care — which are routed to the Clinical Lead / Clinical Advisory Function; and
- **Platform complaints** — concerning the technology, billing, scheduling, or service — handled by Faresay's support/operations function.

12.2 Clinical complaints will be acknowledged within [PLACEHOLDER: timeframe] and investigated by, or under the oversight of, the Clinical Lead, with the Practitioner given a fair opportunity to respond. Outcomes, learning, and any actions (including referral to fitness-to-practise processes where warranted) will be recorded and fed into incident management (Section 11) and quality assurance (Section 10).

12.3 Clients retain the right to complain directly to the Practitioner's Professional Body / licensing board, and Faresay will not obstruct this. Faresay will signpost the relevant body where appropriate. [PLACEHOLDER: complaints contact / address.]

12.4 ⚠️ COUNSEL: Confirm any regulatory complaint-handling requirements, timeframes, and record-keeping obligations applicable in the UK and relevant US States, and how clinical complaints interact with Faresay's terms (`terms-of-service.md`).

13. Suitability / Exclusion Criteria for Onboarding Clients

13.1 The Platform is suitable for adults (and, only if and where Faresay decides to serve them, minors — see below) seeking telehealth mental-health care for presentations that can be safely treated remotely on a cash-pay, planned-care basis.

13.2 ⚠️ CLINICAL: Client suitability screening and exclusion criteria must be defined by the clinical lead and aligned with Section 6 and the Crisis / Safeguarding Policy. Indicative exclusion/triage criteria for onboarding a Client:

- individuals in acute crisis or at imminent risk of harm to self or others (route to emergency services / crisis pathways per `crisis-safeguarding-policy.md`);
- presentations requiring emergency, in-person, or higher-intensity care than telehealth can safely provide (see 6.2);
- individuals located outside a jurisdiction where a suitably-licensed Practitioner is available;
- **minors** — [PLACEHOLDER: whether under-18s are served; if so, age thresholds, parental/guardian consent, and additional safeguards]. ⚠️ CLINICAL / ⚠️ COUNSEL: serving minors materially increases clinical and legal complexity (consent, safeguarding, mandatory reporting); confirm explicitly whether the Platform serves minors at launch.

13.3 Where a prospective or current Client is identified as unsuitable, Faresay and/or the Practitioner will signpost to more appropriate services and follow the escalation steps in the Crisis / Safeguarding Policy where risk is present.

14. Audit and Periodic Review

14.1 Faresay will periodically audit compliance with this Policy, including: a sample audit of Practitioner verification and ongoing-monitoring records (Section 4); review of incident and complaint logs and actions (Sections 11–12); and review of quality-assurance outputs (Section 10).

14.2 This Policy will be reviewed at least [PLACEHOLDER: e.g. annually], and additionally on any material change to law, regulation, professional standards, the business model, or following a serious incident. Review is led by the Clinical Lead and approved by [PLACEHOLDER: approving body].

14.3 Material clinical risks identified through audit and review are recorded in the risk register (`risk-register.md`).

15. Roles and Responsibilities Matrix

⚠️ CLINICAL / ⚠️ COUNSEL: Indicative only. The allocation below — particularly which responsibilities sit with the MSO vs the professional entity ("friendly PC") in US corporate-practice states — must be confirmed with the clinical lead and counsel. [PLACEHOLDER: named roles/owners.]

Area	Practitioner	Clinical Lead	Clinical Advisory Function	Faresay (non-clinical: trust & safety / ops / engineering)
Clinical decisions for individual clients	Owns / accountable	— (no individual-case direction)	—	—
This Policy & clinical standards	Comply	Owns / maintains	Advises / endorses	Supports operationally
Onboarding & verification (Section 4)	Provide evidence	Sets criteria; approves exceptions	Advises	Runs checks operationally
Ongoing licence/sanction monitoring (4.7)	Keep credentials current; disclose	Reviews flags; suspends eligibility	Advises	Runs monitoring; suspends availability operationally
Scope of practice / exclusions (Sections 6, 13)	Works within competence	Defines criteria	Endorses	Implements platform controls
Informed consent (Section 7)	Obtains & documents	Sets standard	Advises	Provides supporting tooling
Clinical records (Section 8)	Owns / holds / retains	Sets minimum standards	Advises	Provides secure platform tooling only
Supervision & CPD (Section 9)	Arranges & evidences	Sets minimum requirement	Advises	Verifies evidence
Quality assurance & outcomes (Section 10)	Participate	Owns programme	Conducts case review	Provides data tooling
Incidents (Section 11)	Report; act on client safety	Reviews / oversees	Reviews serious incidents	Logs; triages

Area	Practitioner	Clinical Lead	Clinical Advisory Function	Faresay (non-clinical: trust & safety / ops / engineering)
Complaints — clinical (Section 12)	Respond	Oversees clinical route	Reviews	Triages; handles platform complaints
Audit & review (Section 14)	Cooperate	Leads	Participates	Supports

16. References to Professional and Ethical Frameworks

16.1 ⚠️ CLINICAL / ⚠️ COUNSEL: The lists below are **indicative** and must be confirmed and completed by the clinical lead and counsel — including which bodies/registers Faresay will recognise, and the correct, current standards for each discipline and jurisdiction.

16.2 United Kingdom (indicative).

- **HCPC** (Health and Care Professions Council) — statutory regulator of practitioner psychologists (and others).
- **BACP** (British Association for Counselling and Psychotherapy) — ethical framework and accreditation for counsellors/psychotherapists.
- **UKCP** (UK Council for Psychotherapy) — register and standards for psychotherapists.
- **BPS** (British Psychological Society) — professional body / practice guidance for psychologists.
- [PLACEHOLDER: other relevant bodies — e.g. BABCP, NCPS — and which Faresay recognises.]

16.3 United States (indicative).

- **State licensing boards** — the primary source of licensure authority and disciplinary rules in each State (psychology, professional counselling, clinical social work, marriage & family therapy).
- **APA** (American Psychological Association) — Ethical Principles of Psychologists and Code of Conduct (psychologists).
- **ACA** (American Counseling Association) — Code of Ethics (counsellors).
- **NASW** (National Association of Social Workers) — Code of Ethics (clinical social workers).
- [PLACEHOLDER: other relevant codes/bodies — e.g. AAMFT for marriage & family therapists.]
- Interstate compacts — **PSYPACT**, the **Counseling Compact**, the **Social Work Licensure Compact** (see Section 4).

16.4 Practitioners remain bound by the ethical and practice standards of the body that registers/licenses them, in addition to this Policy. Where this Policy and a Practitioner's professional standards conflict, the Practitioner must comply with their professional/legal obligations and raise the conflict with the Clinical Lead.

17. Related Documents

- `CONTEXT.md` — shared fact sheet (source of truth).
- `therapist-agreement.md` — Practitioner contractual terms (credentialing, records, fee characterisation).

- `crisis-safeguarding-policy.md` — crisis, risk, safeguarding, and escalation pathways (cross-referenced throughout).
 - `privacy-policy.md` and `security-data-protection-policy.md` — data protection and security.
 - `terms-of-service.md` — client-facing terms.
 - `risk-register.md` — risk capture and tracking.
-

End of DRAFT v0.1. ⚠️ This document must be validated by a qualified clinical lead (and, for the points flagged ⚠️ COUNSEL, by counsel) before use.

Crisis & Safeguarding Policy

⚠️ DRAFT v0.1 — for clinical advisor + legal review. NOT clinical or legal advice. Must be validated by a qualified clinical/safeguarding lead before use. Last updated: 26 June 2026

Faresay — Crisis & Safeguarding Policy

This policy sets out how Faresay and the Therapists who use the Faresay marketplace platform (the "**Platform**") identify, respond to, escalate, and learn from situations involving risk to a Client's safety or the safety of others, and how Faresay discharges its safeguarding responsibilities.

It must be read alongside the [Clinical Governance Policy](#), the [Terms of Service](#), the [Privacy Policy](#), the [Therapist Agreement](#), and the [Security & Data Protection Policy](#). Where this policy and another Faresay document conflict on a safety-critical point, the more protective interpretation applies pending resolution by the Safeguarding Lead.

⚠️ CLINICAL — This document is a structural first draft. Every clinical threshold, screening criterion, escalation step, and exclusion criterion below must be reviewed, corrected, and signed off by a qualified clinical/safeguarding lead before any operational use. Do not rely on it as written.

1. Purpose & Scope

1.1 Purpose. This policy aims to: - (a) protect Clients, Therapists, third parties, and the public from foreseeable serious harm; - (b) make clear that Faresay is **not** an emergency or crisis service, and ensure that message is communicated consistently; - (c) give Therapists a clear, predictable framework for identifying and escalating risk; - (d) define safeguarding roles, responsibilities, and on-call expectations within Faresay; and - (e) ensure incidents are documented, reviewed, and used for learning, consistent with legal and professional obligations.

1.2 Who this applies to. This policy applies to: - all Therapists providing Clinical Services via the Platform (it is incorporated into the [Therapist Agreement](#) by reference — see clauses 6.6–6.7 of that agreement); - all Faresay staff and contractors involved in operations, trust & safety, support, or clinical governance; and - the design and operation of Platform features that touch on safety.

1.3 Territorial scope. Faresay currently operates in the **United Kingdom** and plans to expand into the **United States (all 50 states + DC)** on a cash-pay / out-of-network model (see [CONTEXT.md](#)). This policy covers UK obligations now and forward-looking US obligations for expansion. US-specific provisions are forward-looking and must be finalised before any US launch. ⚠️ COUNSEL / ⚠️ CLINICAL — Many obligations below vary **by US state** and **by UK nation** (England, Scotland, Wales, Northern Ireland); jurisdiction-specific addenda will be required.

1.4 Population scope. This draft assumes the Platform serves **adults only** and that minors are out of scope, consistent with the current assumption in the [Terms of Service](#) and [Therapist Agreement](#). ⚠️ CLINICAL / ⚠️ COUNSEL — If services to minors are ever offered, this policy requires substantial additional child-safeguarding content (consent models, age thresholds — which vary by US state and within the UK, e.g. Gillick/Fraser considerations — verification, and dedicated child-protection escalation). The "safeguarding of children" provisions below currently address (a) risk disclosed about a child who is a third party (e.g. a Client's own child) and (b) future-proofing, not direct treatment of minors.

1.5 Relationship to clinical responsibility. Faresay provides the **technology and marketing platform**; the **Therapist** provides the clinical service and owns the clinical relationship, clinical judgement, and clinical record

(see [CONTEXT.md](#) and [Terms of Service §2](#)). Nothing in this policy transfers clinical responsibility for an individual Client to Faresay or makes Faresay a healthcare provider. The Therapist remains the decision-maker on the clinical care of their Clients, within the framework this policy sets.

2. Faresay Is Not an Emergency or Crisis Service

2.1 Core statement. Faresay and the Platform are **not** an emergency service, crisis line, or suicide-prevention service. Bookings, messages, and other communications sent through the Platform are **not monitored in real time for emergencies**, and Therapists are not available for emergency, urgent, or crisis response. The Platform is intended for **non-urgent, scheduled** mental-health support.

2.2 How this is communicated to Clients. This message must be surfaced clearly and repeatedly, not buried. At minimum (⚠️ CLINICAL / ⚠️ COUNSEL — confirm placement, wording, and frequency): - (a) in the [Terms of Service](#) (see ToS §3, "NOT AN EMERGENCY OR CRISIS SERVICE"), which Clients accept at onboarding; - (b) on the onboarding / intake flow, as a distinct, acknowledged screen — not a buried clause; - (c) persistently within the messaging interface (e.g. a standing banner or footer stating the Platform is not monitored for emergencies and listing crisis resources — see §8); - (d) within any "report a concern" or help surfaces; and - (e) reiterated by the Therapist at the start of the therapeutic relationship as part of informed consent (see [Clinical Governance Policy](#)).

2.3 Wording must be consistent. The crisis-resource wording and numbers used across the Platform, the Terms of Service, and this policy must match. The authoritative list of resources is in §8 of this policy. ⚠️ COUNSEL / ⚠️ CLINICAL — Verify all numbers and service descriptions and keep them synchronised with [Terms of Service §3](#).

2.4 What "not an emergency service" does not mean. This disclaimer does **not** relieve a Therapist of their professional duty of care to a Client during the course of a session or active treatment relationship (see §4), nor of mandatory-reporting and duty-to-warn obligations (see §6–§7). It governs Client expectations of the Platform's availability and monitoring; it does not switch off a clinician's professional and legal duties.

3. Risk Types Covered

This policy covers the following risk presentations. ⚠️ CLINICAL — definitions, indicators, and thresholds for each must be set by the Safeguarding Lead; the descriptions below are placeholders for scope, not clinical guidance.

3.1 Suicidality — suicidal ideation (passive or active), intent, plan, means, and history; acute and chronic risk.

3.2 Self-harm — non-suicidal self-injury and related risk, including escalation patterns and medical risk.

3.3 Risk of harm to others — threats or risk of violence toward identifiable or non-identifiable third parties, including homicidal ideation. Engages possible **duty-to-warn / duty-to-protect** obligations (see §7).

3.4 Domestic abuse — a Client experiencing or perpetrating domestic abuse (physical, sexual, psychological, coercive control, economic, or technology-facilitated abuse). ⚠️ CLINICAL / ⚠️ COUNSEL — Special caution: documentation, safety planning, and any external contact must not increase risk to a Client experiencing abuse; covert tech-facilitated monitoring of the victim must be assumed possible.

3.5 Safeguarding of children — concern that a child (typically a third party, e.g. a Client's child, in this adults-only model) is suffering or at risk of significant harm, abuse, or neglect. Engages **mandatory / statutory reporting** considerations (see §6).

3.6 Safeguarding of vulnerable / at-risk adults — concern that an adult with care-and-support needs is experiencing or at risk of abuse or neglect (sometimes "adult safeguarding"). Engages jurisdiction-specific adult-safeguarding and reporting frameworks (see §6).

3.7 Other acute risk — e.g. acute psychosis, severe disordered eating with medical risk, acute intoxication/overdose risk, severe deterioration. May indicate the presentation is **unsuitable for the Platform** (see

§5 exclusion criteria).

4. Therapist Duty of Care & Responsibilities

4.1 Professional duty. Each Therapist remains bound by the duty of care, ethical codes, and standards of their regulator and professional body (e.g. in the UK: HCPC, BACP, UKCP, BPS, Social Work England / equivalents; in the US: state licensing boards and the relevant professional code). ⚠️ CLINICAL — confirm the applicable bodies and standards per jurisdiction.

4.2 Therapists must: - (a) screen for and remain alert to the risk types in §3 at onboarding and on an ongoing basis (see §5); - (b) hold and maintain competence in risk assessment and management appropriate to the populations they serve, and complete Faresay's required training (see §11); - (c) follow the escalation protocol in §5–§6 when risk is identified; - (d) comply with all mandatory-reporting, duty-to-warn/protect, and safeguarding obligations applicable in the jurisdiction where the Client is located (see [Therapist Agreement §6.6](#)); - (e) maintain confidentiality, disclosing only as permitted or required by law or to protect against serious harm (see §7 and [Therapist Agreement §6.5](#)); - (f) document risk, concerns, decisions, and actions contemporaneously in the clinical record (see §9); - (g) carry their own professional indemnity / malpractice insurance as required by the [Therapist Agreement](#); and - (h) not take on, and promptly escalate/offboard, Clients whose presentation falls within the exclusion criteria (§5).

4.3 Scope of the Therapist's role in crisis. The Therapist is responsible for clinical risk decisions for their own Clients. The Therapist is **not** expected to provide emergency response in place of statutory emergency services; the appropriate action in an acute emergency is to direct the Client to emergency services and, where duties require and it is safe and lawful, to make contact with emergency or relevant authorities (see §5).

4.4 Availability and limits. Therapists must make their availability and response-time limits clear to Clients (the Platform is not monitored for emergencies; messages are not answered in real time). ⚠️ CLINICAL — define expected message-response windows and how Therapists communicate their boundaries.

5. Client Risk Screening, Ongoing Assessment & Exclusion Criteria

5.1 Screening at onboarding. Before or at the start of the therapeutic relationship, risk-relevant information should be gathered to determine whether the Platform and the matched Therapist are an appropriate setting for the Client. ⚠️ CLINICAL — The Safeguarding/Clinical Lead must define the screening instrument(s), questions, scoring, and decision rules. At minimum, screening should consider current and historical suicidality/self-harm, risk to others, acute psychiatric presentation, substance use, and the Client's location (for emergency routing and licensure — see §8.3 and [CONTEXT.md](#)).

5.2 Ongoing assessment. Risk is dynamic. Therapists must reassess risk throughout the relationship, in particular at transitions (intake, deterioration, major life events, medication changes, ending of therapy) and whenever risk indicators appear in session or in messages.

5.3 Exclusion criteria — presentations unsuitable for cash-pay telehealth. Certain presentations are **not suitable** for delivery via an unscheduled-monitoring, remote, cash-pay marketplace and should be excluded or referred to a higher level of care. ⚠️ CLINICAL — the Clinical/Safeguarding Lead must finalise this list; this cross-references and must remain consistent with the suitability/exclusion provisions in the [Clinical Governance Policy](#). Indicative (placeholder) examples to be confirmed: - (a) acute, high, or imminent risk of suicide or serious self-harm requiring crisis or inpatient care; - (b) current intent or plan to seriously harm another person; - (c) acute psychosis, mania, or severe decompensation requiring urgent/specialist or in-person care; - (d) severe substance dependence requiring medically supervised detox/withdrawal; - (e) severe eating disorders with medical instability; - (f) presentations requiring a level of monitoring, frequency, or coordination the Platform cannot safely provide; - (g) [PLACEHOLDER: other criteria to be defined by the Clinical/Safeguarding Lead].

5.4 Action where exclusion criteria apply. Where a Client's presentation meets an exclusion criterion, the Therapist must not continue routine treatment as if the setting were appropriate; instead they must (in line with clinical judgement and §6 escalation): support the Client to access an appropriate level of care, signpost crisis/emergency resources (§8), document the decision (§9), and notify Faresay's Safeguarding Lead where required. ⚠️ CLINICAL / ⚠️ COUNSEL — abandonment risk: ending or declining care must be handled to avoid clinical abandonment and to meet professional/ethical "termination of care" rules in the jurisdiction (coordinate with [Clinical Governance Policy](#) and [Therapist Agreement](#) continuity-of-care provisions).

5.5 Matching and licensure gate. A Therapist must be licensed in the jurisdiction where the Client is located (UK; and in the US, the relevant state — leveraging PSYPACT / Counseling Compact / Social Work Licensure Compact per [CONTEXT.md](#)). Location is also needed for emergency routing (§8.3). ⚠️ COUNSEL — confirm licensure verification gates by jurisdiction.

6. Crisis Identification & Step-by-Step Escalation Protocol

⚠️ CLINICAL — This protocol is a **structural placeholder**. The Clinical/Safeguarding Lead must define the actual clinical decision thresholds, who does what, timeframes, and jurisdiction-specific contacts before any operational use. The numbered steps below describe the *shape* of the process, not validated clinical instructions.

6.1 Identify. A Therapist (or, where a Platform safety signal is triggered, Faresay's Trust & Safety function) identifies indicators of acute risk (§3) — disclosed directly, observed in session, or surfaced via a risk flag (§8.2).

6.2 Assess severity and immediacy. The Therapist assesses whether the risk is: - **Imminent / life-threatening** (e.g. active attempt in progress, immediate intent and means, immediate danger to a child or another person); or - **Elevated but not immediately life-threatening**; or - **Lower-level / chronic** but requiring monitoring and planning. ⚠️ CLINICAL — define indicators and thresholds for each tier.

6.3 Immediate-danger pathway (imminent/life-threatening). - (a) Prioritise getting the Client/affected person to emergency help **now**: direct them to call emergency services (US **911**; UK **999**) or, where safe and within the Therapist's lawful authority and professional duty, contact emergency services on their behalf using the Client's location (see §8.3). - (b) Surface and provide crisis resources (§8). - (c) Where duty-to-warn/protect or mandatory-reporting obligations are engaged, follow §6.5 / §7 / §6.6. - (d) Stay with / maintain contact to the extent clinically appropriate and possible within the session. - (e) Notify Faresay's Safeguarding Lead / on-call per §6.7 and document per §9. ⚠️ CLINICAL / ⚠️ COUNSEL — Contacting emergency services without consent, and using location data to do so, has clinical, legal, and confidentiality implications that vary by jurisdiction; confirm lawful basis (and see the emergency data-sharing carve-out, §8.4).

6.4 Elevated-risk pathway (not immediately life-threatening). - (a) Conduct/complete a structured risk assessment and collaborative safety plan with the Client. - (b) Signpost crisis resources (§8) and agree what the Client will do if risk escalates. - (c) Consider increased session frequency, coordination with the Client's GP/primary care or other providers (with consent where required), and whether exclusion criteria (§5.3) now apply. - (d) Document and, where thresholds are met, escalate to the Safeguarding Lead (§6.7) and follow reporting duties (§6.6/§7).

6.5 Duty-to-warn / duty-to-protect trigger (risk to others). Where a Client presents a serious risk of harm to an identifiable third party, the Therapist must follow the duty-to-warn / duty-to-protect framework applicable in the Client's jurisdiction (see §7). ⚠️ COUNSEL — this is **state-variable in the US** and threshold-based in the UK.

6.6 Mandatory-reporting trigger (children / vulnerable adults). Where information indicates a child or vulnerable/at-risk adult is suffering or at risk of significant harm, the Therapist must follow the mandatory/statutory reporting framework applicable in the Client's jurisdiction (see §6 reporting detail in §7) and notify the

Safeguarding Lead. ⚠️ COUNSEL / ⚠️ CLINICAL — reporting duties, recipients, and timeframes vary by US state and UK nation.

6.7 Internal escalation to Faresay. In parallel with clinical action, the Therapist notifies Faresay's **Safeguarding Lead / on-call contact** of a serious incident via [PLACEHOLDER: defined reporting channel — e.g. dedicated email, phone, in-app report], within [PLACEHOLDER: timeframe]. Faresay's role here is **supportive and governance-focused** (logging, support, pattern detection, learning, regulatory/notification obligations) — not to override the Therapist's clinical decisions for an individual Client. ⚠️ CLINICAL — define the threshold for mandatory internal escalation, the channel, and the timeframe.

6.8 Platform safety signals. Where a Platform feature (e.g. a self-harm keyword flag, §8.2) raises a signal independent of a Therapist, define how Trust & Safety triages it, what is automated vs human-reviewed, and how false positives and Client privacy are handled. ⚠️ CLINICAL / ⚠️ COUNSEL — automated risk detection on sensitive mental-health content carries clinical, false-negative/positive, privacy, and liability risks; confirm before building.

7. Confidentiality Breach Thresholds, Duty-to-Warn & Mandatory Reporting

7.1 General principle. Client confidentiality is a core duty (see [Therapist Agreement §6.5](#), [Privacy Policy](#)). It may be **lawfully overridden only** where permitted or required to prevent or reduce a risk of serious harm, or where the law requires disclosure. Disclosure should be the **minimum necessary** to address the risk, made to the appropriate recipient, and documented (§9).

7.2 United States — duty to warn / duty to protect (Tarasoff line of authority). ⚠️ COUNSEL — **State-variable.** Following *Tarasoff* and its progeny, many US states impose a duty to warn and/or protect identifiable third parties from a Client's serious threats; some states make it mandatory, some permissive, some have no statute, and the trigger, the discharge mechanism (warn the victim, notify police, initiate hospitalisation, etc.), and the immunity provisions differ **state by state**. The applicable rule is generally that of the state where the Client/Therapist is situated. Counsel must produce a **per-state matrix** of duty-to-warn/protect obligations and safe-harbour steps before US launch, and the Platform/training must reflect each state's rule.

7.3 United Kingdom — confidentiality-breach thresholds. ⚠️ COUNSEL / ⚠️ CLINICAL — UK practitioners may breach confidentiality to prevent serious harm, broadly under common-law public-interest disclosure, data-protection conditions for processing special-category data, and professional-body guidance (e.g. GMC/BACP/HPC-aligned principles, ICO guidance). Counsel/clinical lead must define the threshold ("real and serious risk of serious harm"), who may be told (e.g. police, the at-risk person, GP), and how to record the justification — and note variation across England, Scotland, Wales, and Northern Ireland.

7.4 Mandatory reporting — children. ⚠️ COUNSEL / ⚠️ CLINICAL — In the US, child-abuse mandatory-reporting laws apply to therapists in **all states but differ by state** (who must report, what triggers it, to whom, and timeframes). In the UK, the framework is statutory and nation-specific (e.g. local-authority children's services / multi-agency safeguarding hubs; police; relevant working-together guidance). Counsel/clinical lead must specify the recipient, trigger, and timeframe for each jurisdiction.

7.5 Mandatory / permitted reporting — vulnerable & at-risk adults. ⚠️ COUNSEL / ⚠️ CLINICAL — US adult-protective-services reporting duties vary by state (some mandatory for certain professionals, some permissive). In the UK, adult-safeguarding duties run through local-authority adult social care (e.g. Care Act 2014 in England and equivalents in the other nations). Specify recipients, triggers, and timeframes per jurisdiction.

7.6 Domestic abuse. ⚠️ COUNSEL / ⚠️ CLINICAL — Reporting and information-sharing in domestic-abuse contexts is sensitive and jurisdiction-specific; disclosure can increase danger to a victim. Define safe practice, lawful basis, and any mandatory elements per jurisdiction; do not contact a victim's household or share information in ways that could be intercepted by an abuser.

7.7 **Conflict of duties.** Where the obligations of the Client's jurisdiction, the Therapist's licence, professional ethics, and this policy appear to conflict, the Therapist should apply the most protective lawful course and seek guidance from the Safeguarding Lead and, where time permits, their own professional/legal advisers. ⚠️ COUNSEL.

8. Emergency Resources & Safety Features

8.1 **Crisis resources to surface.** The following resources must be displayed in the surfaces described in §2.2. ⚠️ COUNSEL / ⚠️ CLINICAL — **Verify, localise, and keep synchronised with Terms of Service §3.**

United States - 988 Suicide & Crisis Lifeline — call or text **988**. - **911** — for any life-threatening emergency. - [PLACEHOLDER: state/territory-specific or population-specific crisis resources to be added per jurisdiction.]

United Kingdom - 999 — for any life-threatening emergency. - **111** — for urgent (non-life-threatening) NHS help. - **Samaritans** — free on **116 123** (24 hours a day). - [PLACEHOLDER: nation-specific resources / text services to be added — e.g. text-line services; confirm before publication.]

If the Client is outside these countries, direct them to their local emergency services. ⚠️ CLINICAL — define handling for Clients located outside supported jurisdictions.

8.2 **Risk flags.** The Platform may provide features to flag risk (e.g. a Therapist-set risk indicator on a Client record, or detection signals on messages). ⚠️ CLINICAL / ⚠️ COUNSEL — Define what flags exist, who sets/sees them, how they affect care and matching, how privacy is protected, and the risk of automated detection (false positives/negatives). Any automated scanning of clinical/message content must have a confirmed lawful basis and be assessed for clinical safety before deployment.

8.3 **Location capture for emergencies.** To enable emergency routing (correct emergency number; ability to direct or send help to the right place; correct duty-to-warn/reporting jurisdiction), the Platform should capture the Client's **location/jurisdiction**. ⚠️ COUNSEL / ⚠️ CLINICAL — Confirm what location data is collected (self-declared jurisdiction vs precise geolocation), when, lawful basis, retention, and how it is used in an emergency; precise real-time geolocation has significant privacy implications and must be assessed in the DPIA / privacy review (see [Privacy Policy](#) and [Security & Data Protection Policy](#)).

8.4 **Emergency data-sharing carve-out.** In a life-at-risk emergency, Faresay and/or the Therapist may need to share limited personal data (e.g. identity, location, contact details, nature of risk) with emergency services or relevant authorities **without prior consent** to protect the vital interests of the Client or another person. This carve-out must be reflected in, and operated consistently with, the [Privacy Policy](#) and the [Security & Data Protection Policy](#). ⚠️ COUNSEL — Confirm the lawful basis (e.g. UK GDPR vital-interests / public-interest / legal-obligation conditions for special-category data; and, in the US, HIPAA's permitted disclosures to avert a serious and imminent threat and state-law equivalents), the minimum-necessary principle, and logging requirements.

9. Documentation & Record of Concerns

9.1 **Clinical record.** The Therapist must contemporaneously document risk assessments, safety plans, disclosures, decisions (including the reasoning for any confidentiality breach or report), actions taken, and who was contacted, in the **clinical record** they own (see [CONTEXT.md](#), [Therapist Agreement](#)). ⚠️ CLINICAL — define minimum documentation standards and templates.

9.2 **Faresay record of concerns.** Faresay should maintain a secure **safeguarding incident log / record of concerns** capturing reported serious incidents, escalations, and outcomes, accessible only to authorised safeguarding/governance personnel. ⚠️ CLINICAL / ⚠️ COUNSEL — define what Faresay records vs what stays in

the clinician's clinical record (to respect the controller/processor split in [Privacy Policy §1](#)), access controls, and retention.

9.3 Retention & security. Records of concern are highly sensitive special-category data and must be retained and secured per the [Privacy Policy](#) and [Security & Data Protection Policy](#), for the retention period set there. ⚠️
COUNSEL — confirm retention periods (UK + US state law).

9.4 Auditability. Escalations, reports, and emergency data-sharing decisions should be time-stamped and auditable to support post-incident review (§10) and any regulatory inquiry.

10. Safeguarding Leads, Roles & On-Call Expectations

10.1 Safeguarding Lead. Faresay will designate a **Safeguarding Lead** (and a **Deputy**) with defined responsibilities for this policy, incident oversight, escalation support, liaison with authorities where appropriate, and learning. ⚠️
CLINICAL — The Safeguarding Lead should be (or be advised by) a suitably qualified clinical/safeguarding professional. Confirm the required qualification, the appointment, and named contacts. – **Safeguarding Lead:** [PLACEHOLDER: name / role / contact] – **Deputy Safeguarding Lead:** [PLACEHOLDER: name / role / contact] – **Clinical Lead / Advisor:** [PLACEHOLDER: name / role / contact] (see [Clinical Governance Policy](#))

10.2 Trust & Safety function. [PLACEHOLDER: team/role] triages Platform safety signals and non-clinical reports and escalates to the Safeguarding Lead per §6.8.

10.3 On-call expectations. ⚠️ CLINICAL / ⚠️ COUNSEL — Decide and document whether Faresay offers any out-of-hours safeguarding contact, the response-time expectation, and the limits of that contact. **Critically:** an on-call channel must **not** be presented to Clients as an emergency or crisis line — doing so would contradict §2 and could create reliance and liability. Any internal on-call is for Therapist/staff escalation and governance, not Client emergency response. Confirm scope, staffing, and SLAs.

10.4 Limits of Faresay's role. Faresay does not provide clinical care, does not supervise the clinical judgement of independent Therapists, and is not a substitute for emergency services. Its safeguarding role is governance, support, logging, learning, and meeting its own legal/regulatory obligations.

11. Therapist Training Requirements

11.1 Required training. Before onboarding and periodically thereafter, Therapists must complete and attest to Faresay's required training on this policy, covering: identifying and assessing the risk types in §3; the escalation protocol (§6); confidentiality-breach thresholds, duty-to-warn/protect, and mandatory reporting in the jurisdiction(s) they serve (§7); use of Platform safety features (§8); and documentation (§9). ⚠️ CLINICAL — define the curriculum, format, assessment, and refresh cadence.

11.2 Jurisdiction-specific knowledge. Therapists must be familiar with the duty-to-warn/protect and mandatory-reporting rules of every jurisdiction in which they hold themselves out to treat Clients. ⚠️ COUNSEL / ⚠️ CLINICAL — Faresay should provide a per-jurisdiction reference (the §7.2 matrix) but the Therapist remains professionally responsible.

11.3 Competence. Therapists must only accept Clients within their competence and must keep risk-management skills current per their professional body's CPD requirements. ⚠️ CLINICAL.

11.4 Records. Faresay should record training completion and attestations as part of credentialing (see [Clinical Governance Policy](#)).

12. Post-Incident Review & Learning

12.1 **Review of serious incidents.** Serious safeguarding/crisis incidents should be reviewed by the Safeguarding Lead (with the Clinical Lead and, where appropriate, the Therapist) to identify what happened, whether this policy and the escalation protocol were followed, what worked, and what should change. ⚠️ CLINICAL — define what counts as a "serious incident" requiring review, and the review method and timeframe.

12.2 **Learning loop.** Findings should feed back into training (§11), this policy, the [Clinical Governance Policy](#), screening/exclusion criteria (§5), and Platform safety features (§8). Themes and trends should be reviewed periodically.

12.3 **Support & a just culture.** Reviews should support a just, learning-oriented culture (supporting Clients, Therapists, and staff affected by a serious incident) rather than being purely punitive, while preserving accountability where standards were not met. ⚠️ CLINICAL.

12.4 **External duties.** ⚠️ COUNSEL / ⚠️ CLINICAL — Confirm any external reporting/notification duties arising from a serious incident (e.g. to regulators, professional bodies, data-protection authorities for any associated breach, or coroner/medical-examiner processes), per jurisdiction.

13. Review Cadence

13.1 This policy will be reviewed at least **annually**, and additionally after any serious incident (§12), after any material change in law or professional guidance, and before entering a new jurisdiction. ⚠️ CLINICAL / ⚠️ COUNSEL — confirm cadence and owner.

13.2 **Owner:** [PLACEHOLDER: Safeguarding Lead]. **Next review due:** 26 June 2026.

13.3 **Version control.** Material changes should be versioned and the "Last updated" date refreshed. This is **DRAFT v0.1** and must not be operationalised until validated by a qualified clinical/safeguarding lead and reviewed by counsel.

⚠️ DRAFT v0.1 — Not for operational use. Validate with a qualified clinical/safeguarding lead (⚠️ CLINICAL) and legal counsel (⚠️ COUNSEL) before publication or reliance. Keep crisis resources synchronised with the [Terms of Service](#).